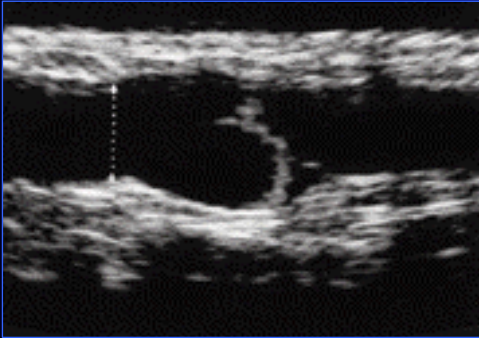


L'insuffisance Veineuse et les Varices

Jean-Pierre Laroche
CHU de Montpellier

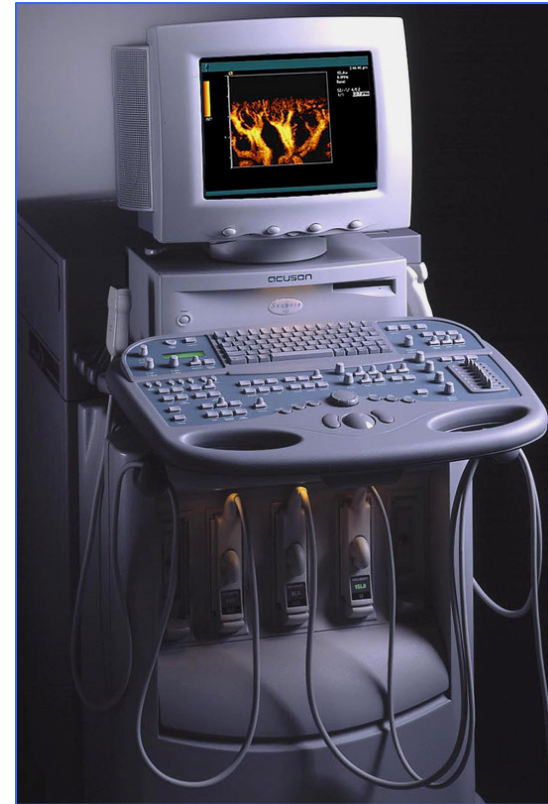
2^{ème} Partie

Varices



Reflux

Examen clinique et instrumental



Recommendations

EXPLORATION VEINEUSE ULTRASONORE DANS LA MALADIE VEINEUSE CHRONIQUE DES MEMBRES INFERIEURS. CONFERENCE DE CONSENSUS DE L'UIP.

1^{ère} partie : principes de bases.

Duplex Ultrasound Investigation of the Veins in Chronic Venous Disease of the Lower Limbs – UIP Consensus. Document Part 1 : Basic Principles

**P. Coleridge-Smith,^{1*} N. Labropoulos,² H. Partsch,³ K. Myers,⁴ A. Nicolaides⁵
and A. Cavezzi ⁶**

*London, UK; ²Chicago, IL, USA; ³Vienna, Austria; ⁴Melbourne, Australia; ⁵Nicosia, Cyprus;
and ⁶San Benedetto del Tronto, Italy*

Eur J Vasc Endovasc Surg 2006 ; 31 : 83 – 92

www.sfmv.fr

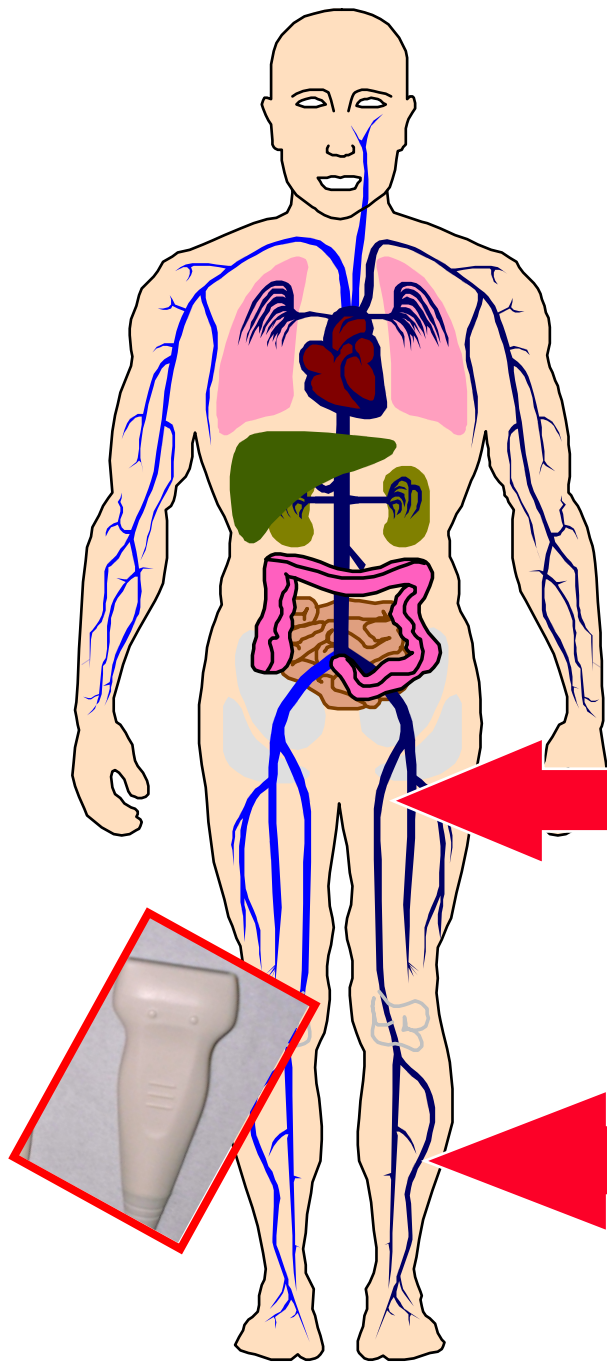
Examen d'un sujet variqueux

- MG
- 1. Interrogatoire : rechercher un facteur prédisposant, un facteur héréditaire, un facteur aggravant, un antécédent de TVP ou TVS.
 - 2. Examen clinique : tests classiques + Doppler de poche à la recherche de reflux
 - 3. Doppler Couleur : étude réseau profond (rapide) puis affinement de l'approche clinique au niveau superficiel
- S



Escabeau de phlébologie

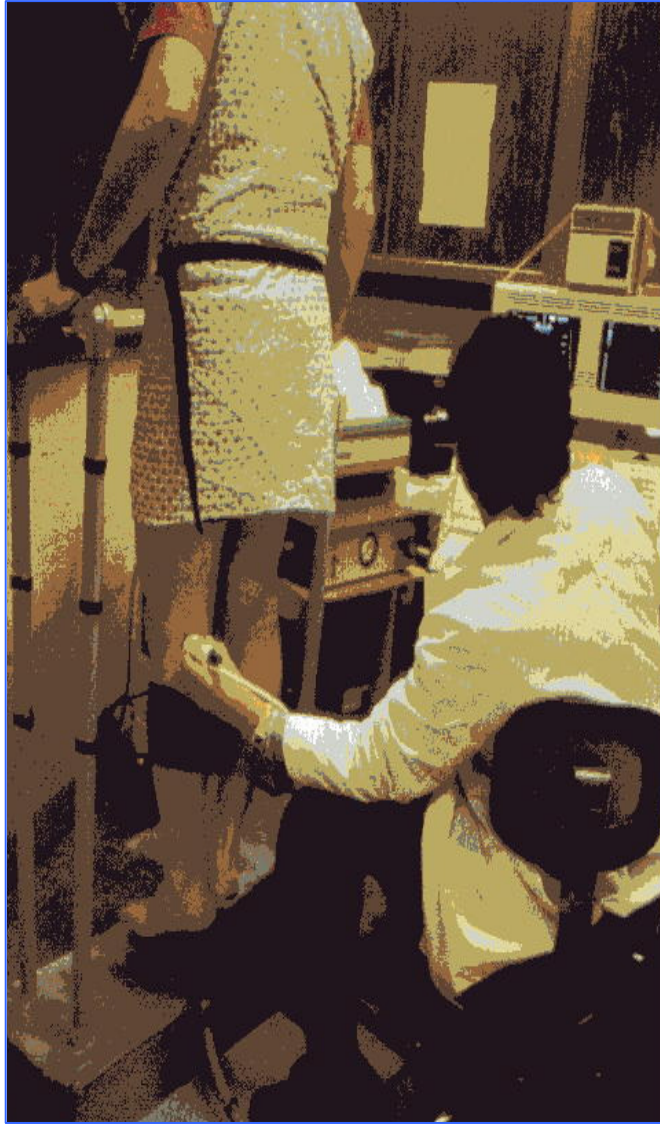
Examen d'un sujet variqueux ORTHOSTATISME



Sonde



**Compression musculaire
(distale)**

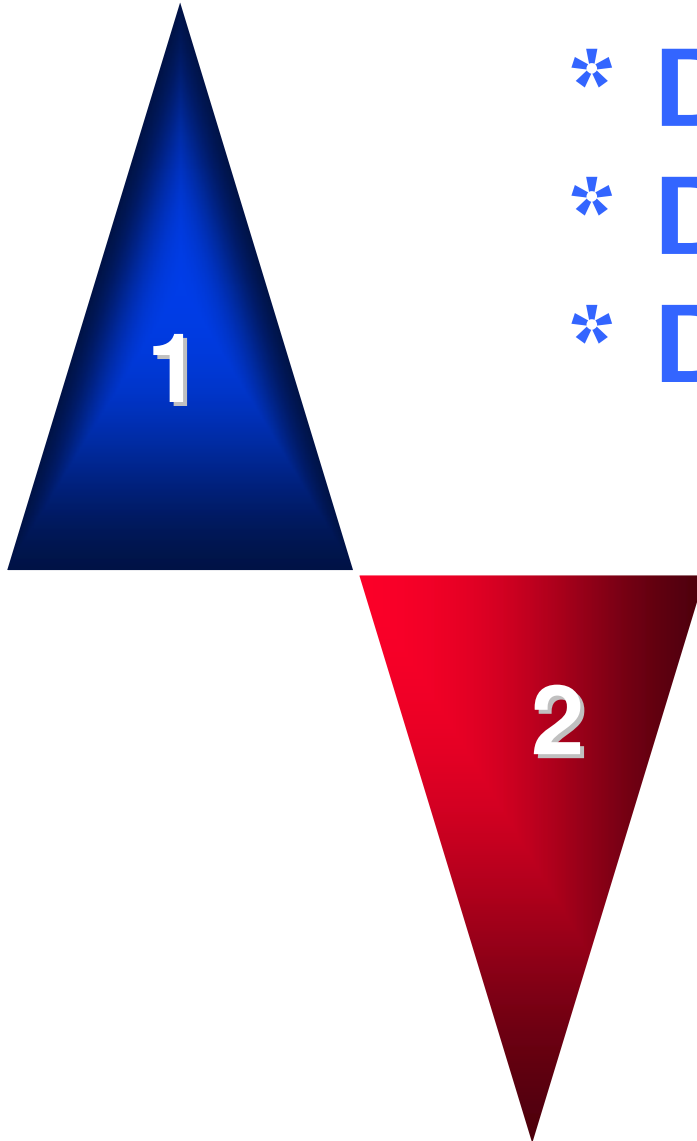


Cuff



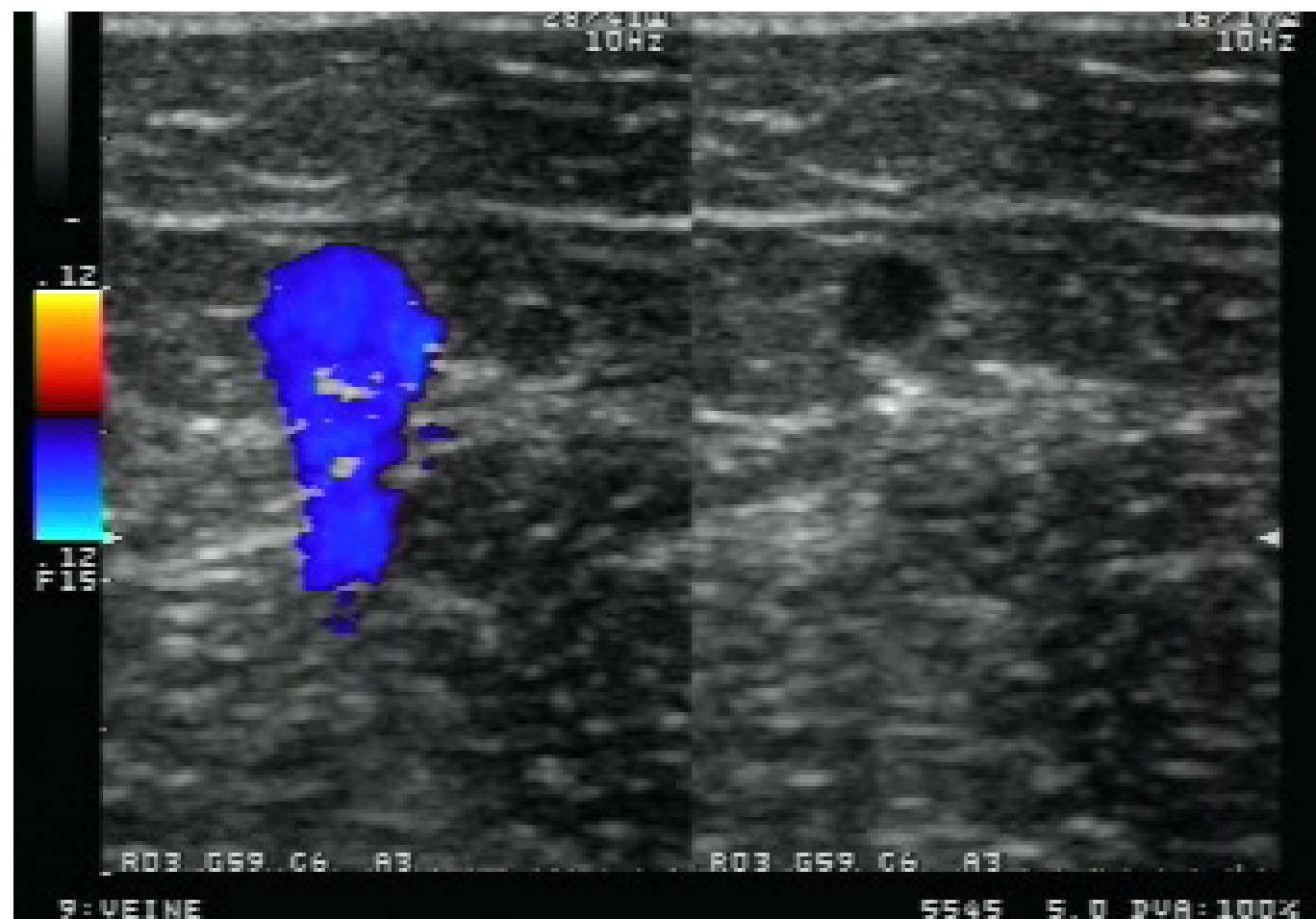
Reflux veineux

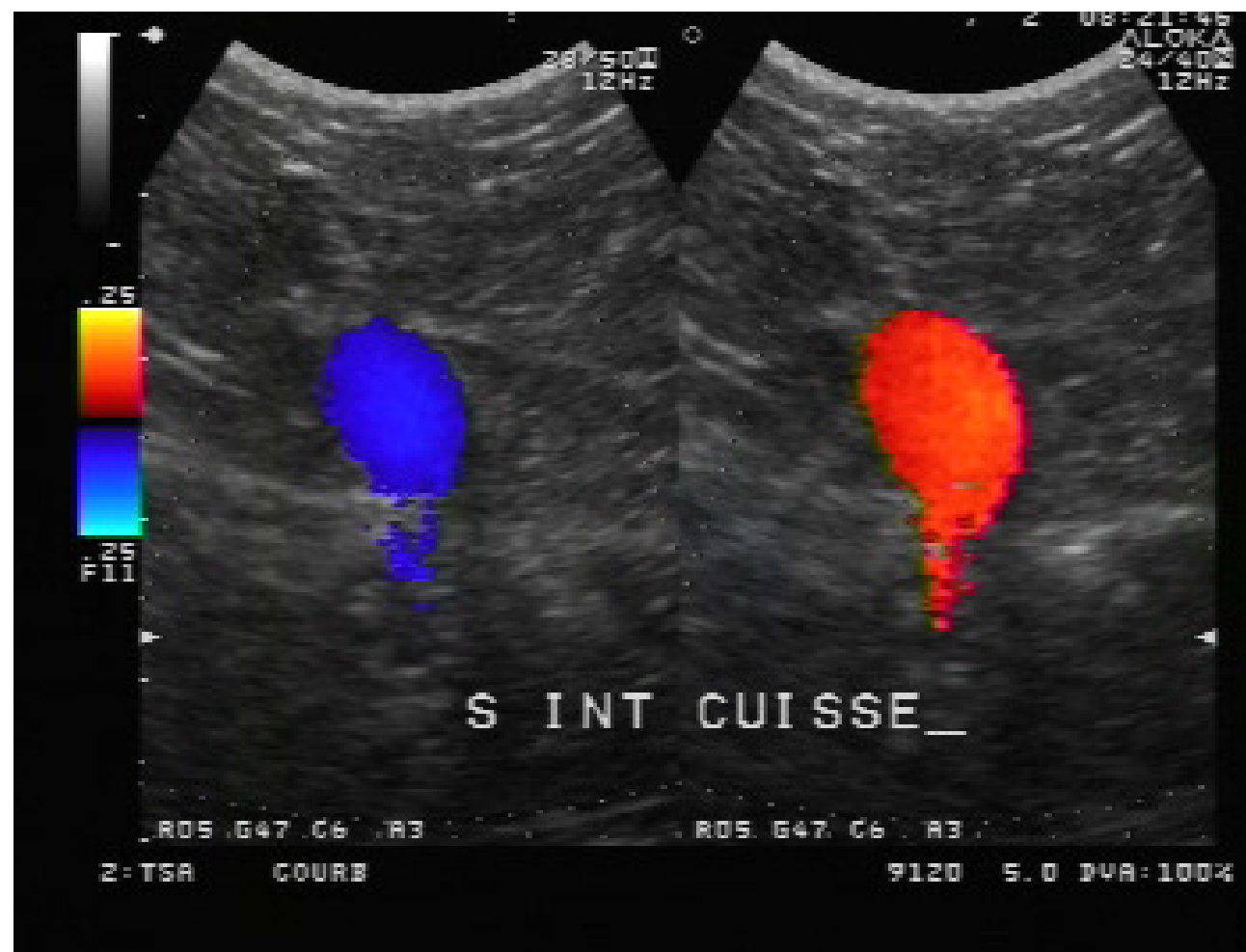
- * Doppler continu
- * Doppler pulsé
- * Doppler couleur



1 : Flux

2 : Reflux





ANGIO SYNERGIE 84

Y

10-05-'04

3 10:26:32

94/100
8Hz

31/38
8Hz

.20

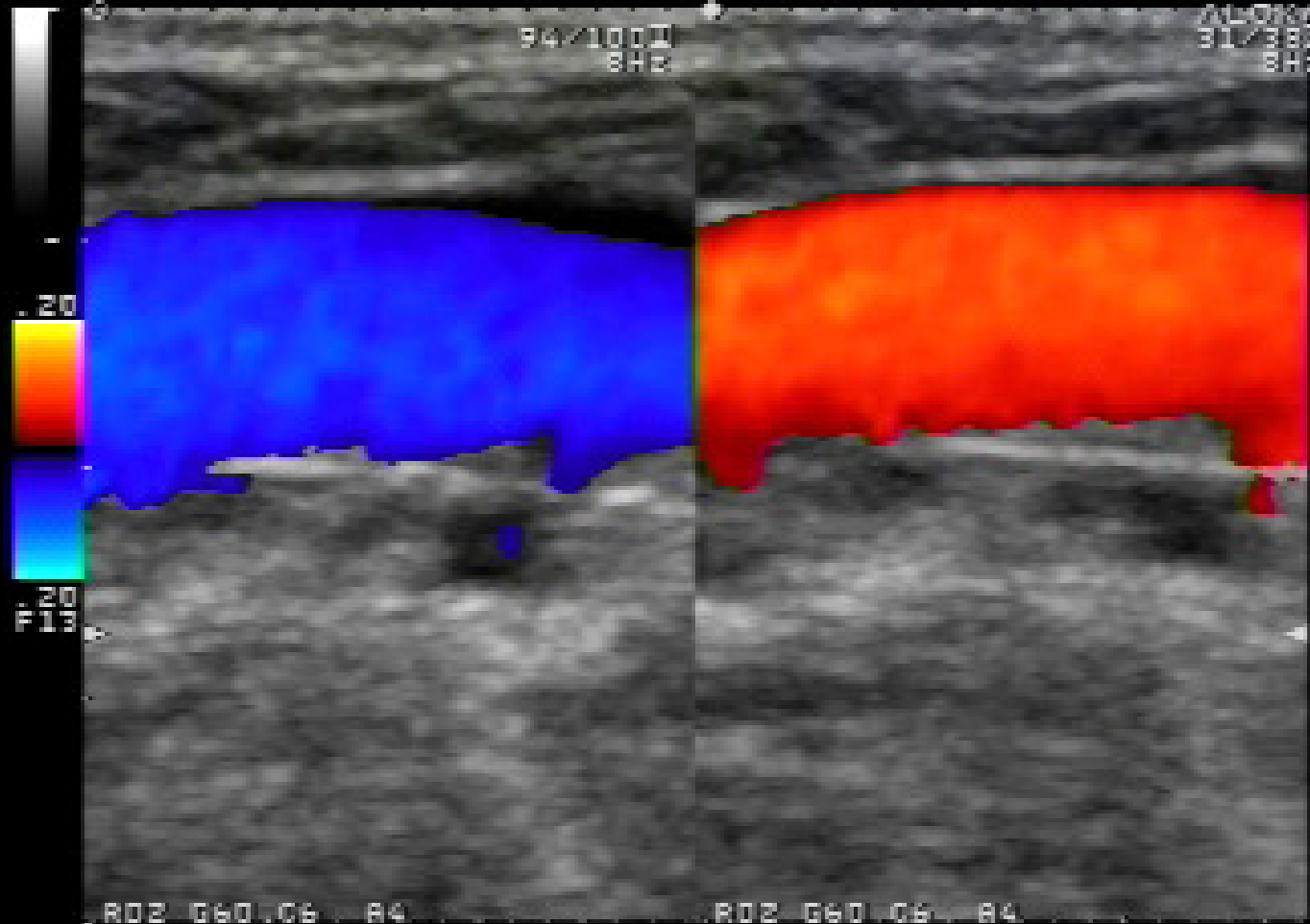
.20
13

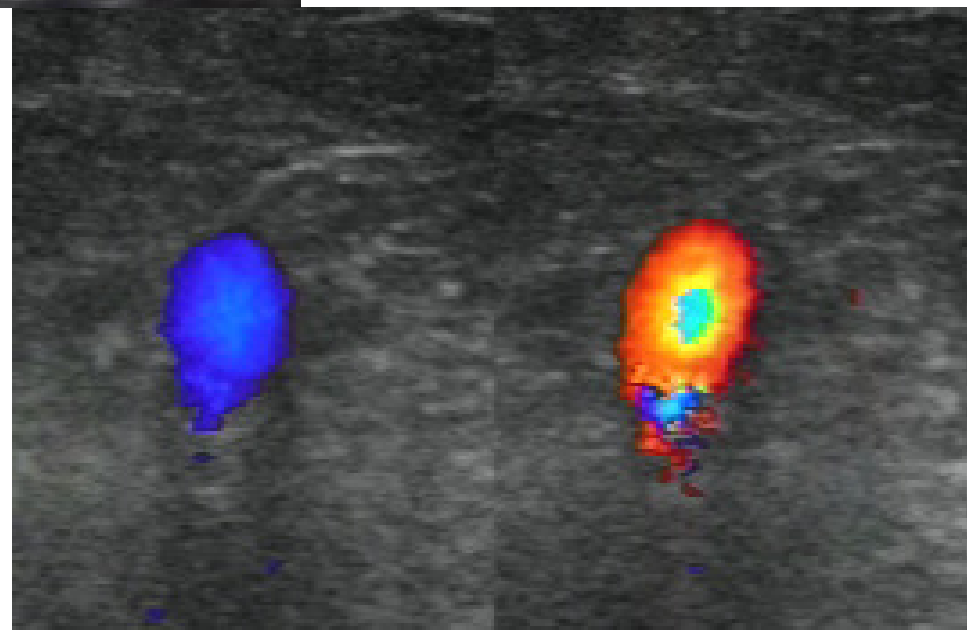
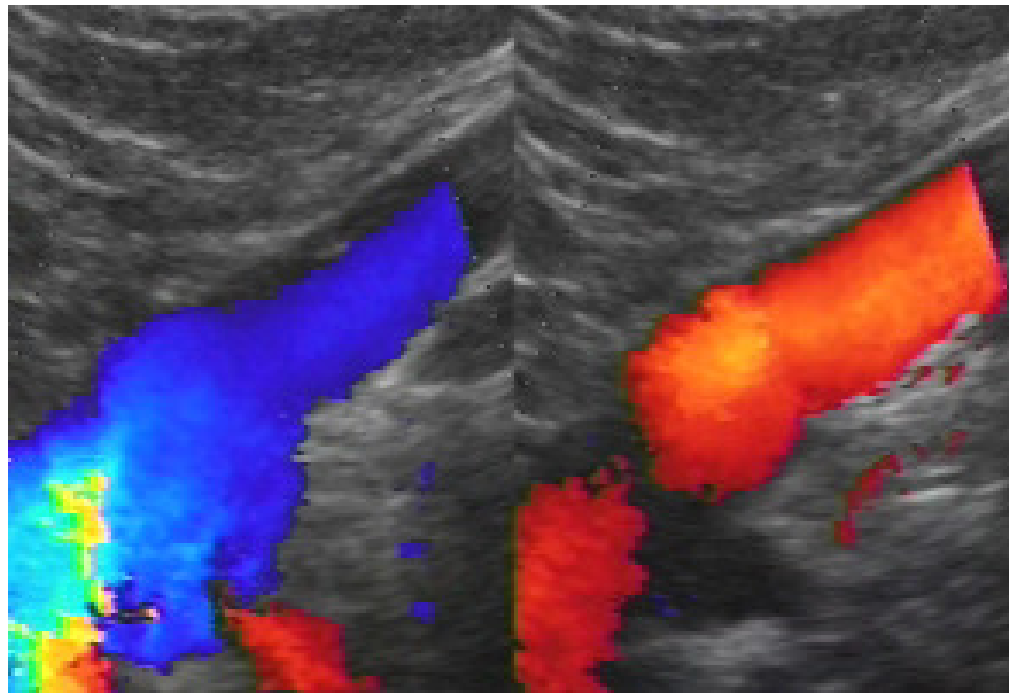
RDZ G60 C6 A4

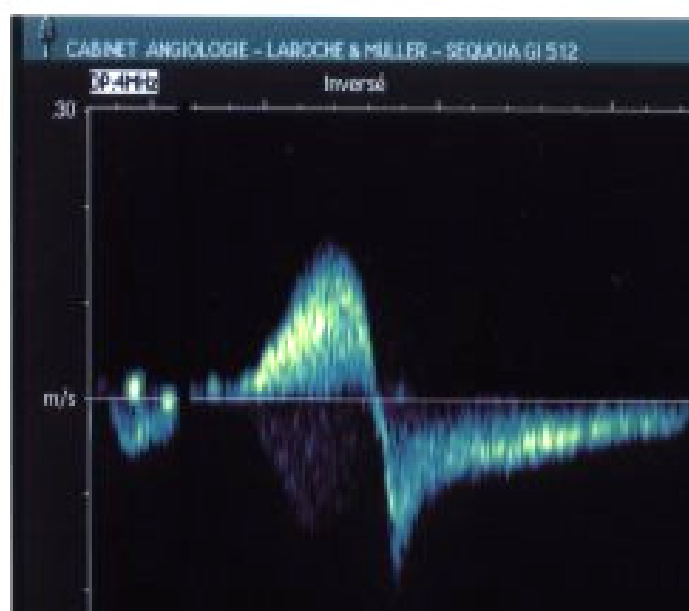
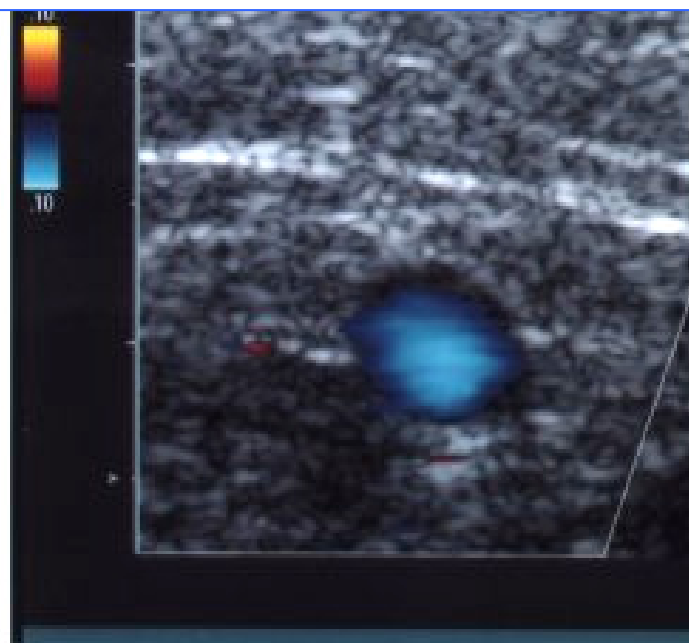
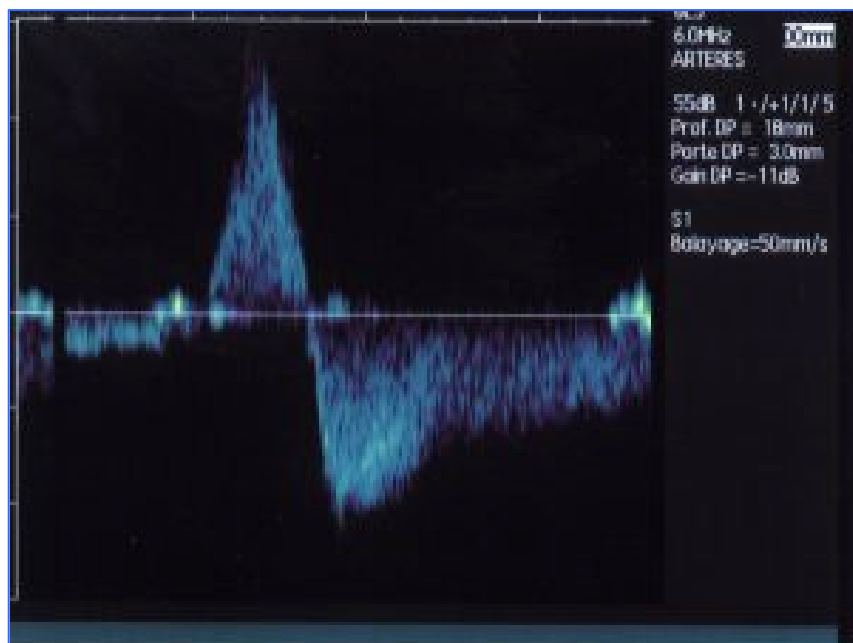
RDZ G60 C6 A4

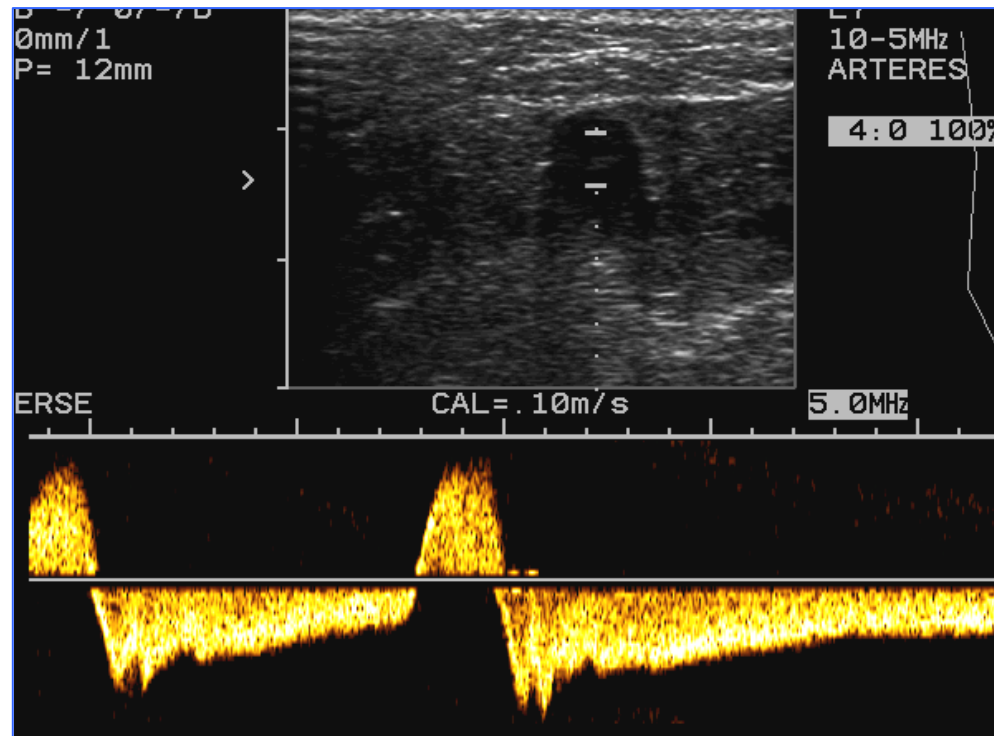
1: T. S. A.

5545 5.0 PWR:100%

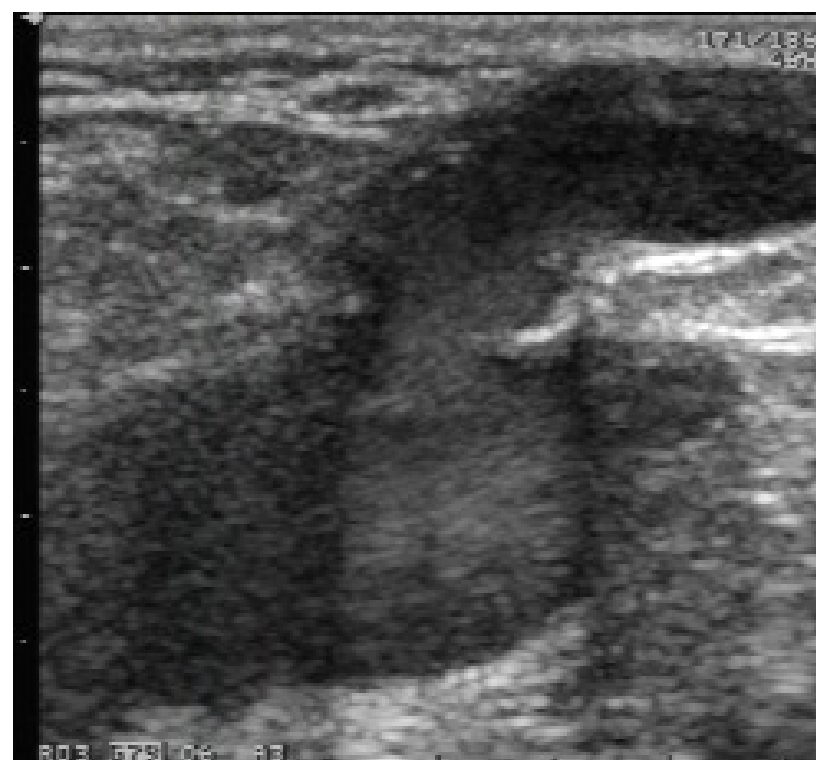
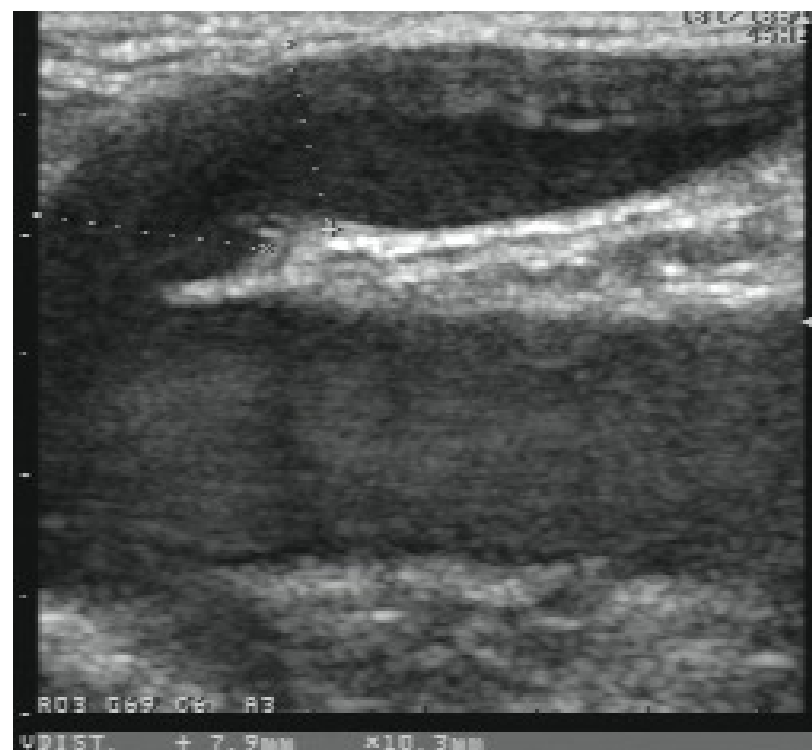




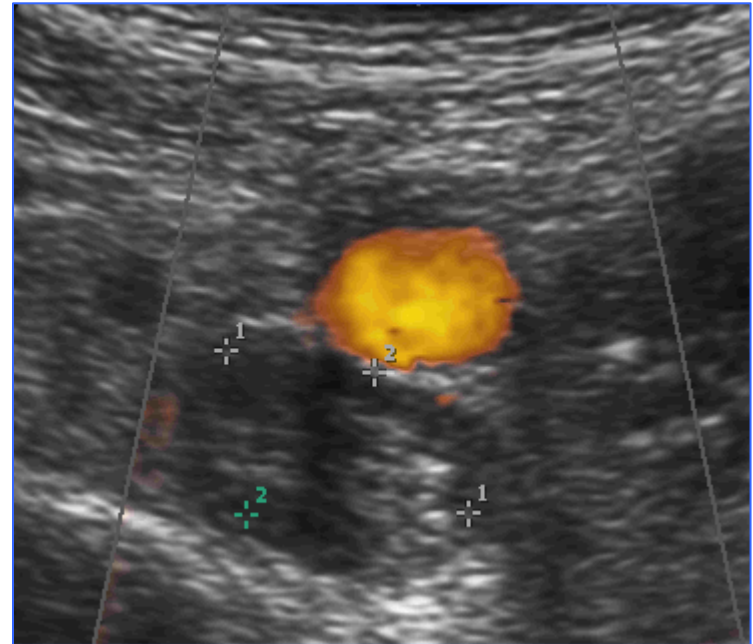


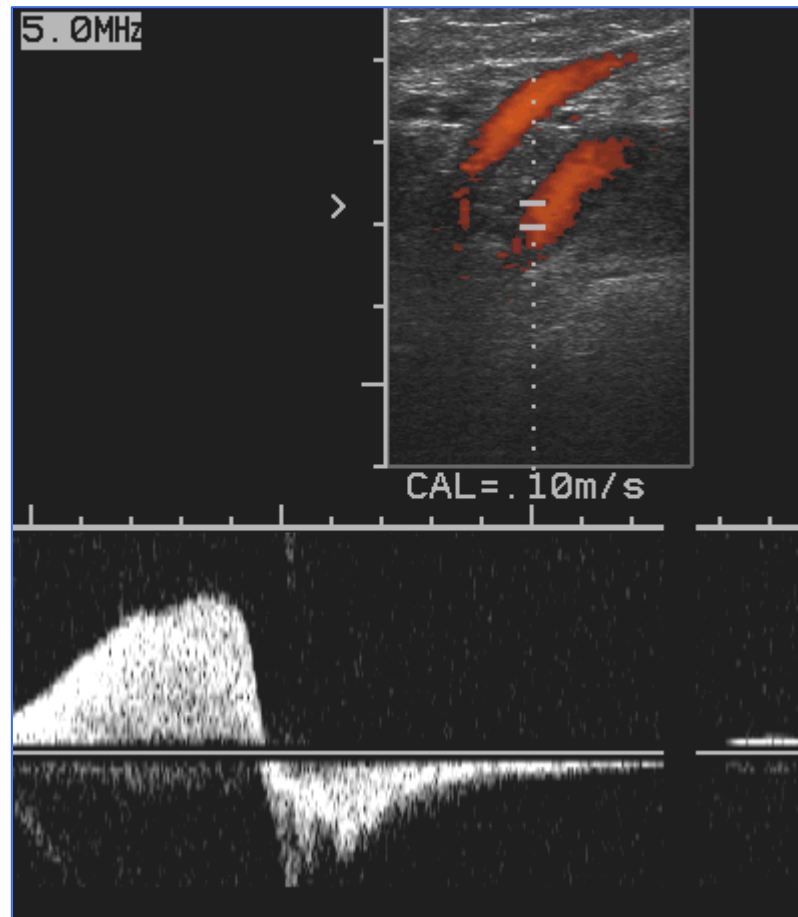
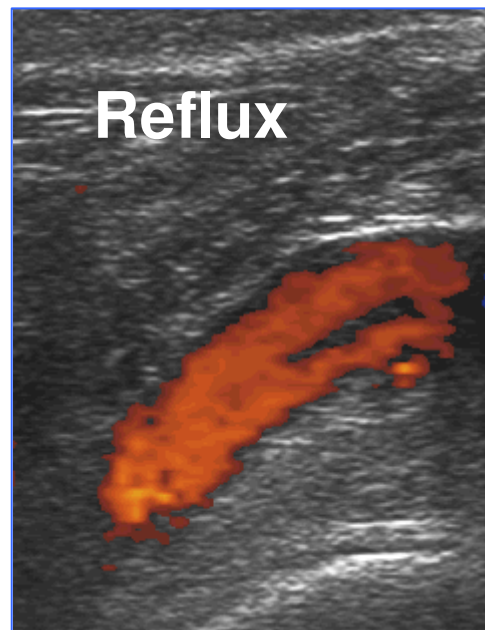
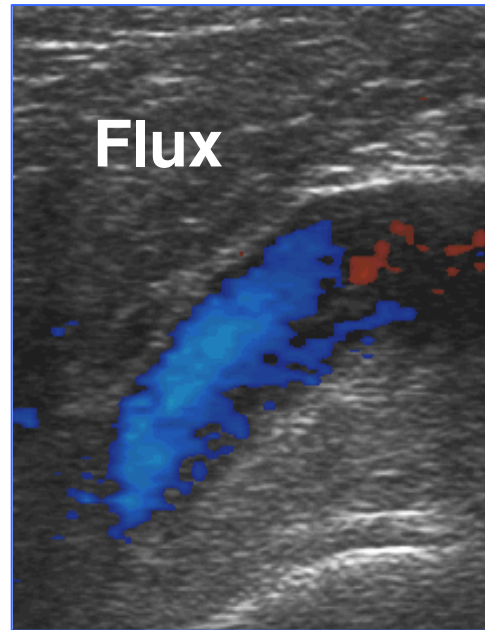


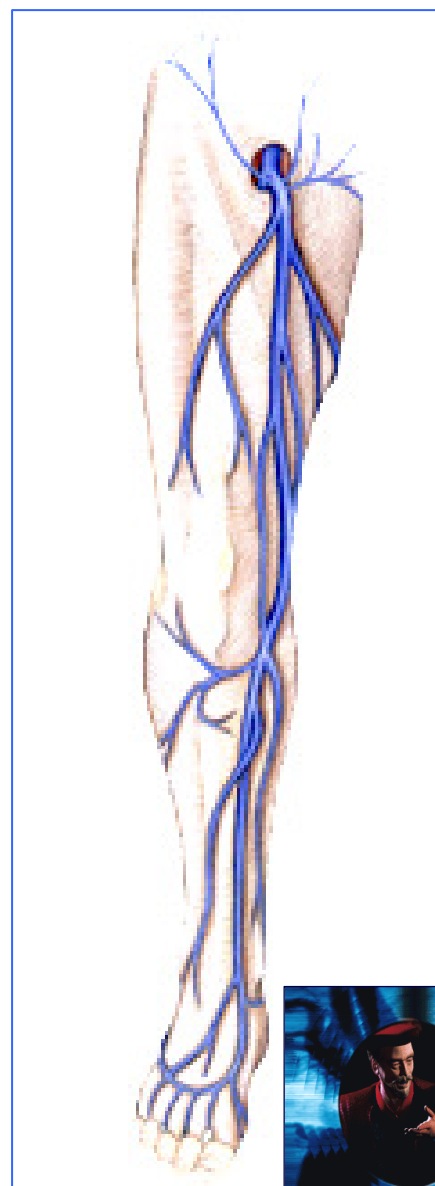
***Reflux veine grande saphène
au Doppler pulsé : Reflux = signature
hémodynamique de l'atteinte valvulaire***

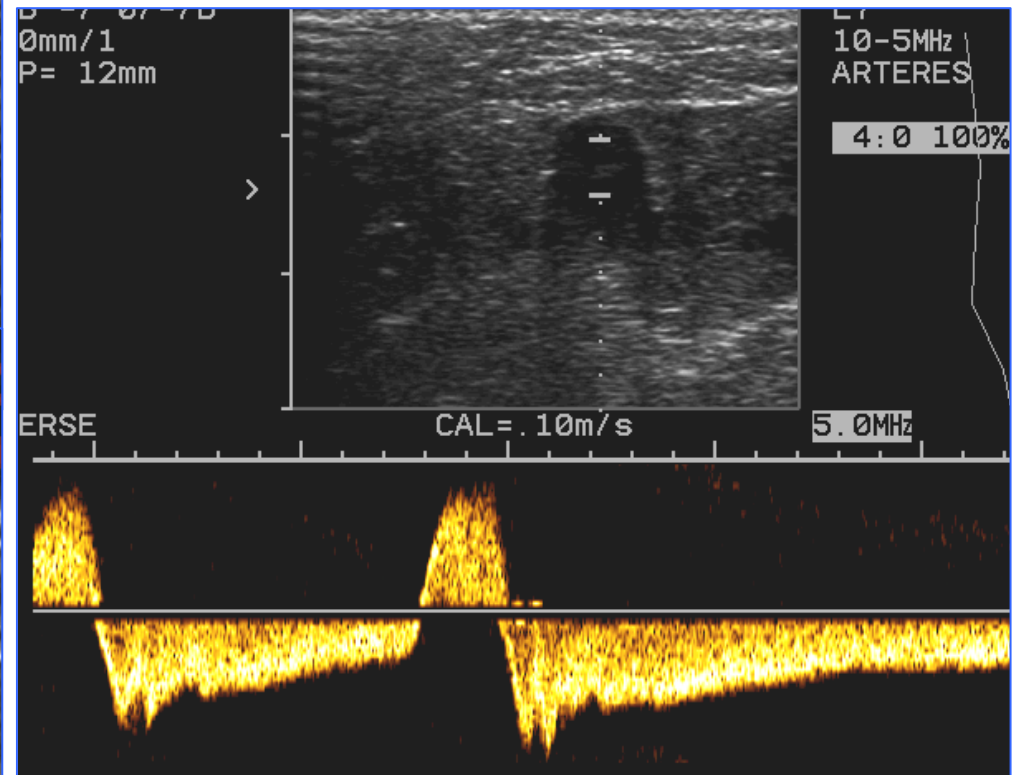
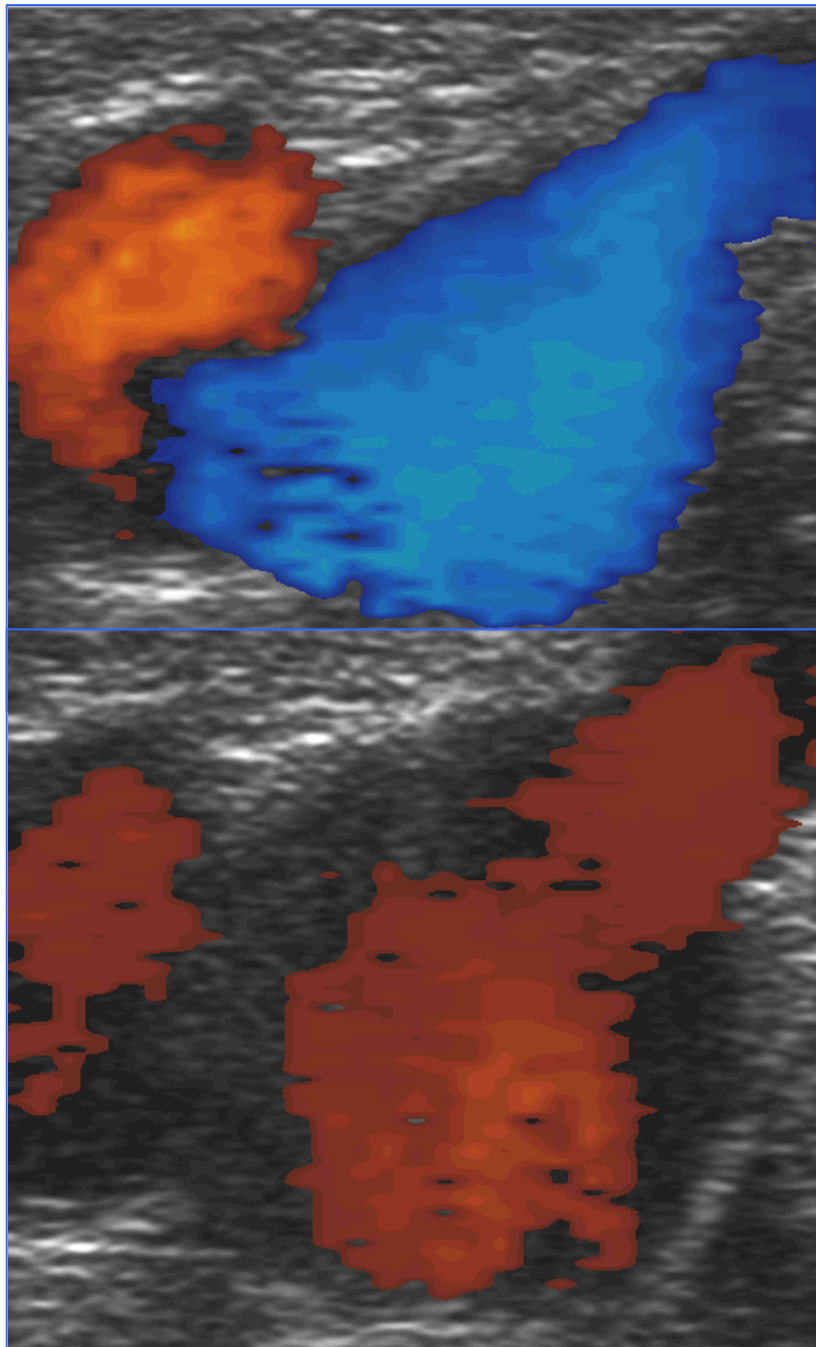


**Attention au réseau
veineux profond !**

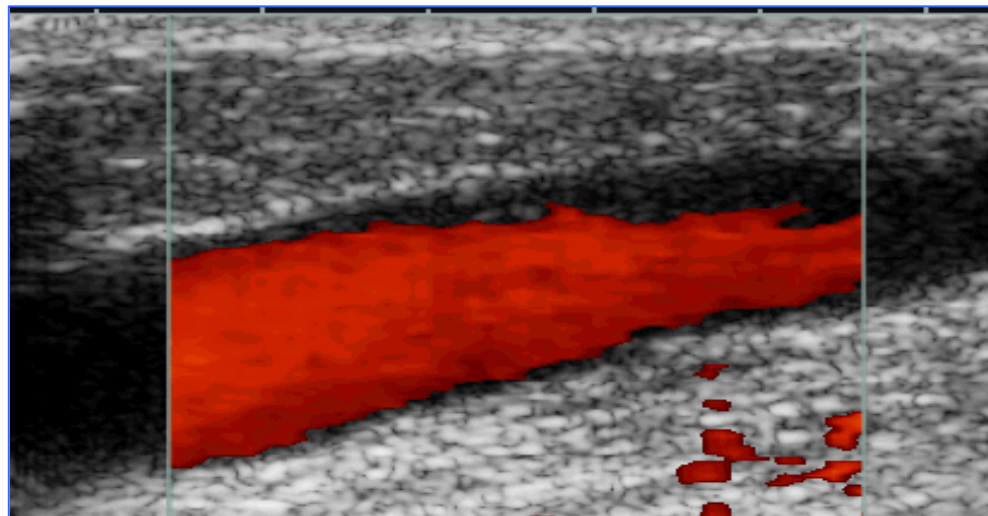
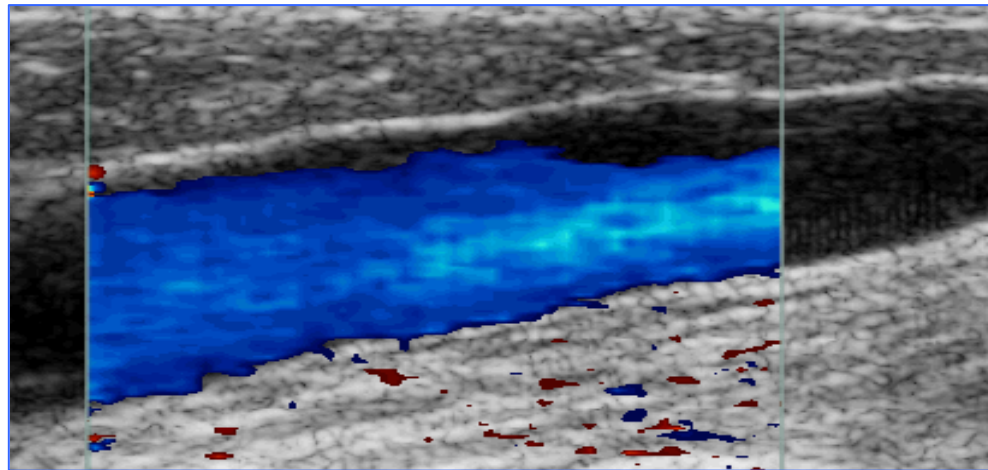




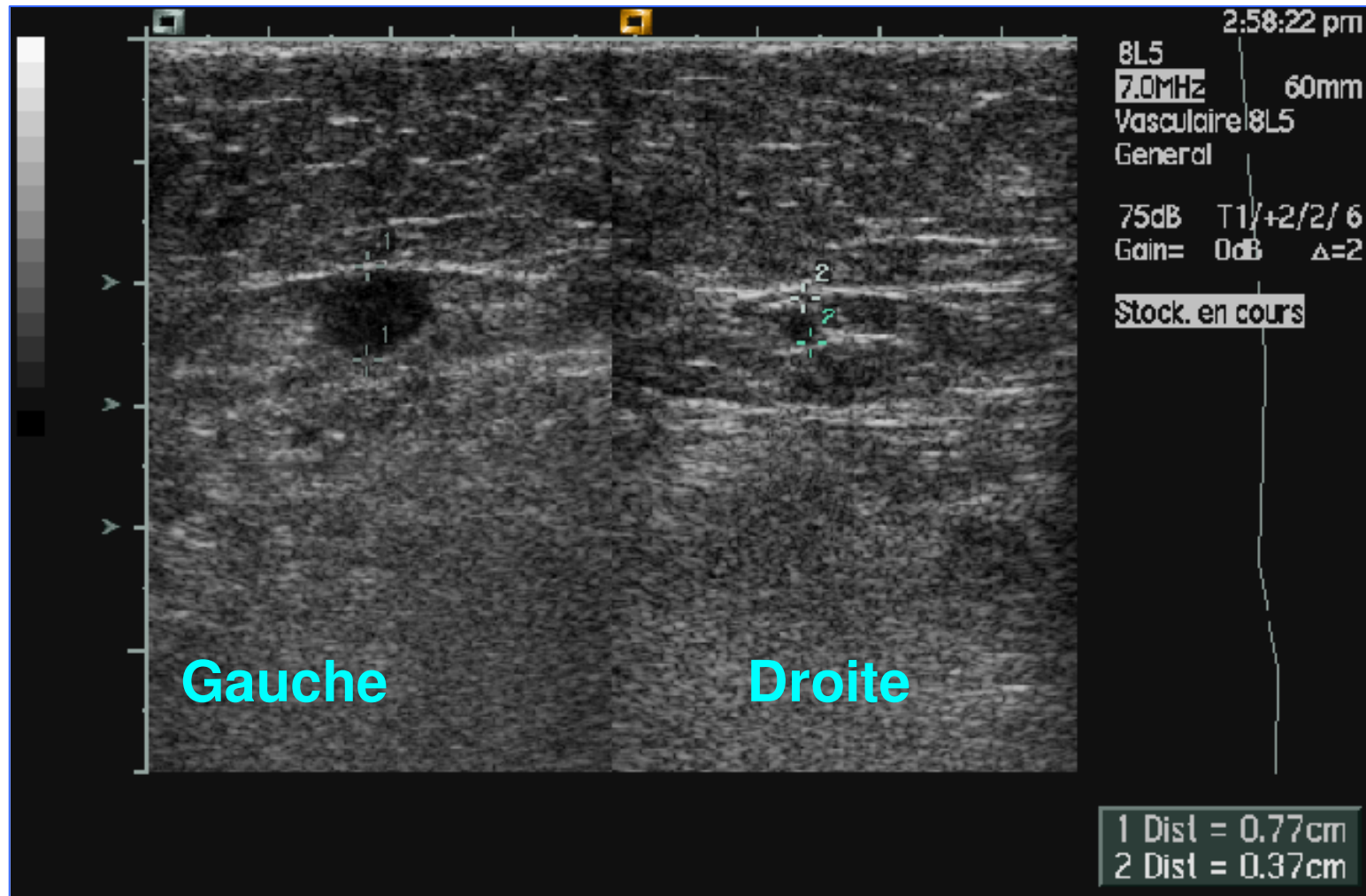




*Reflux grande saphène
ostial et tronculaire*



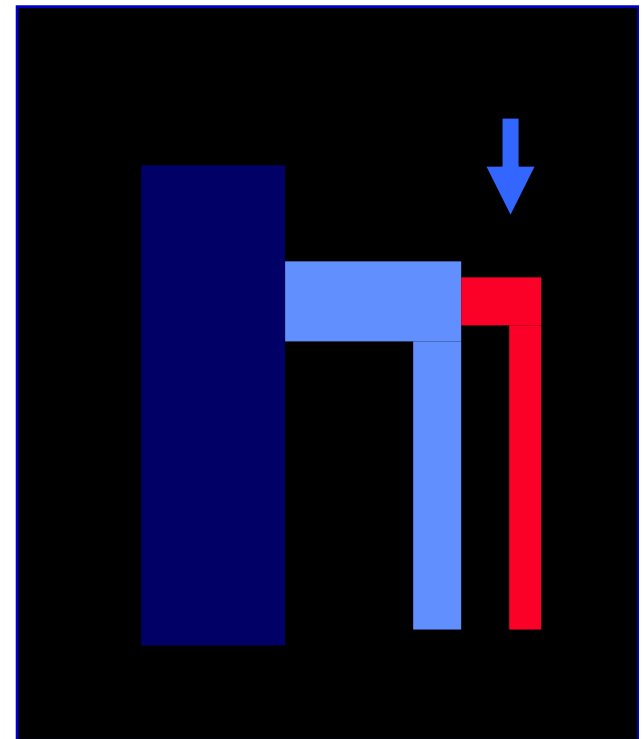
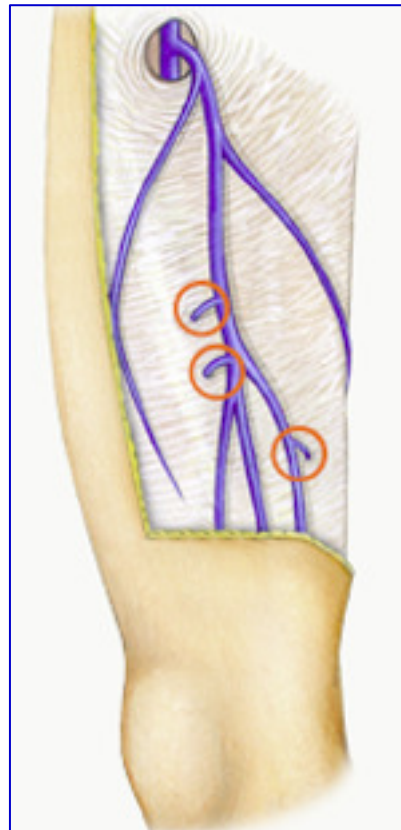
Flux / Reflux veine grande saphène



Veine grande saphène, veine continente
à droite, incontinente à gauche en Doppler pulsé
diamètre augmenté à gauche

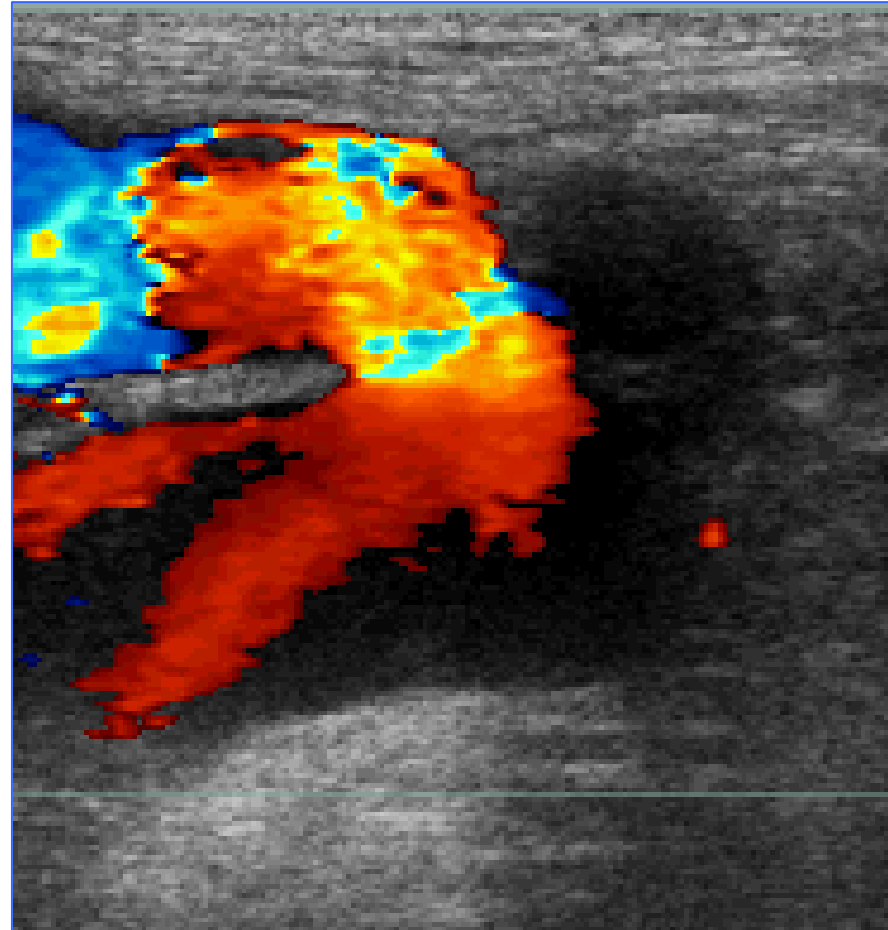
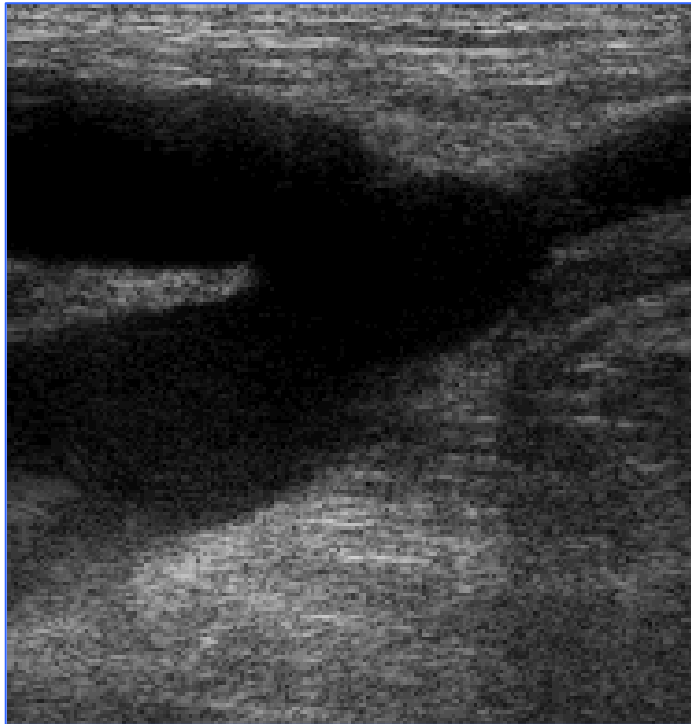
Saphène antérieure

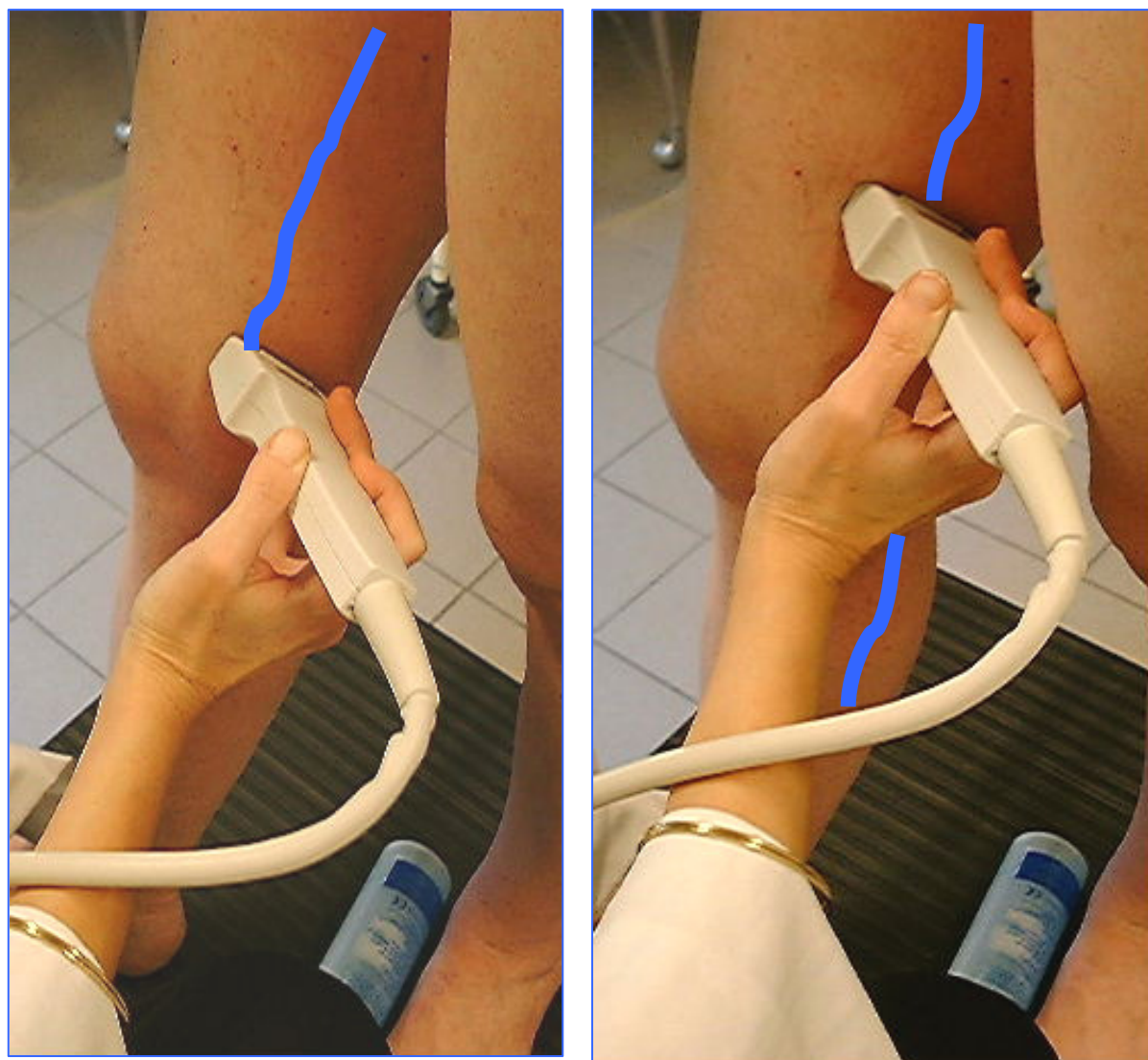
- Collatérale crosse grande saphène
- Peut présenter un reflux isolé ++++.



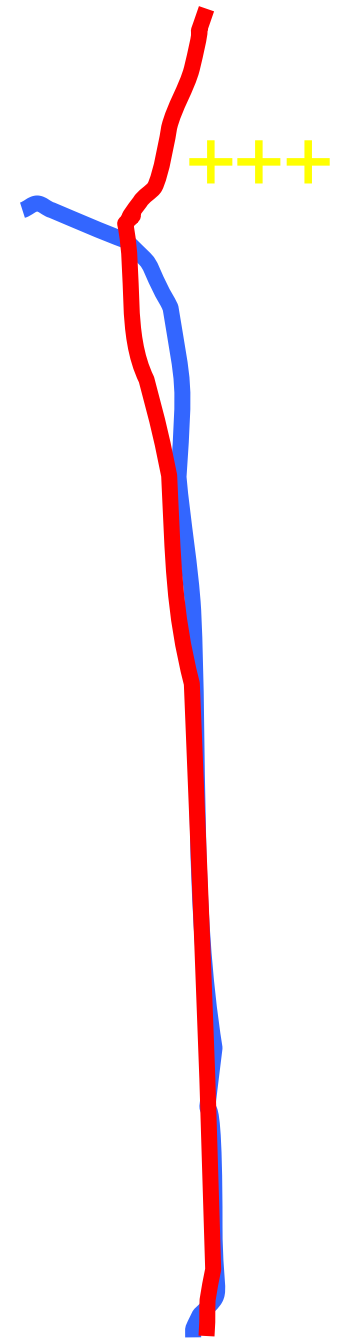
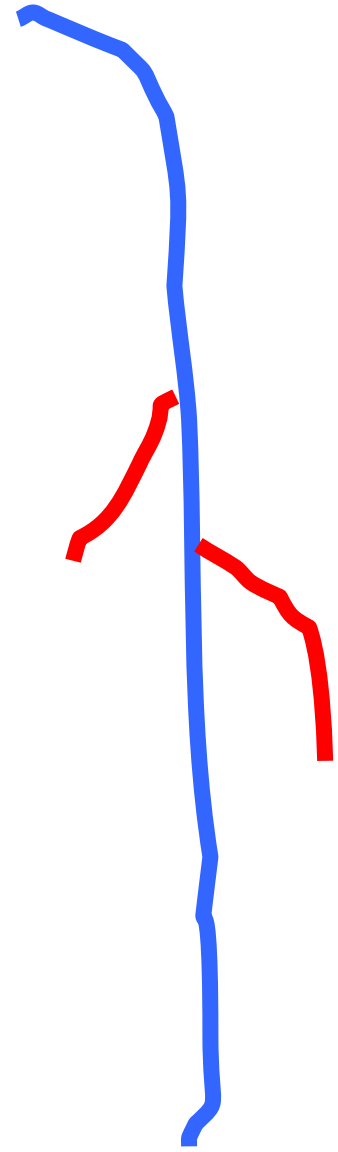
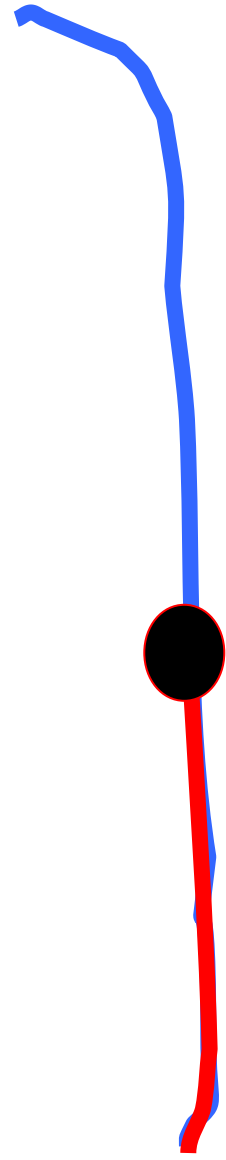
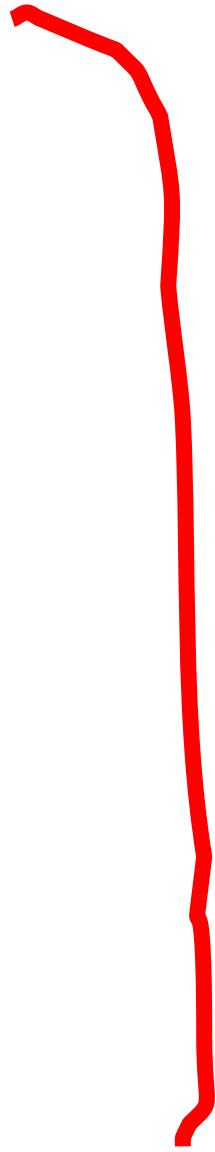
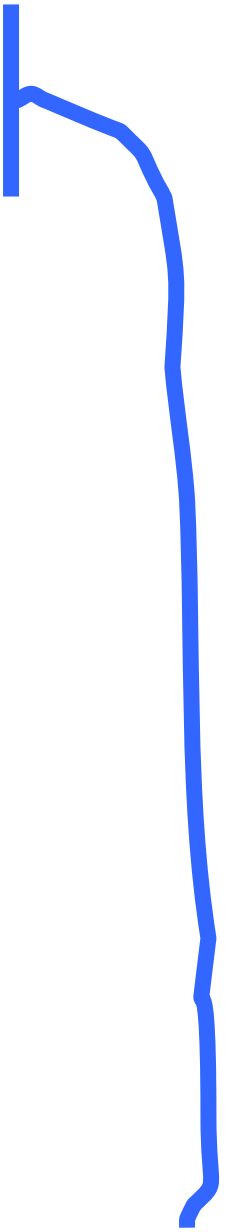


Saphène antérieure



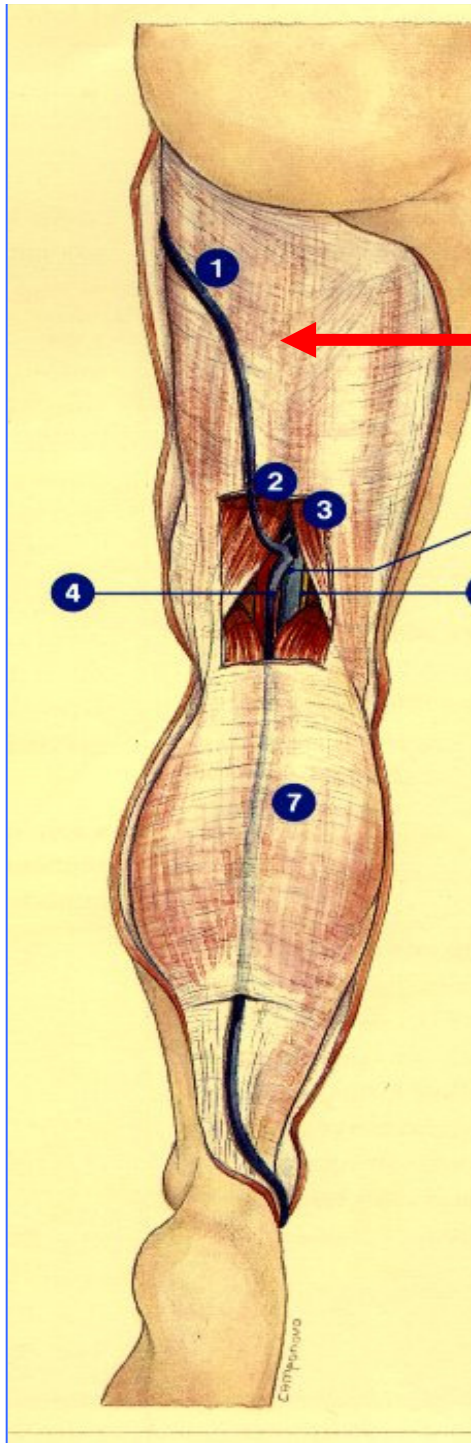


Grande saphène

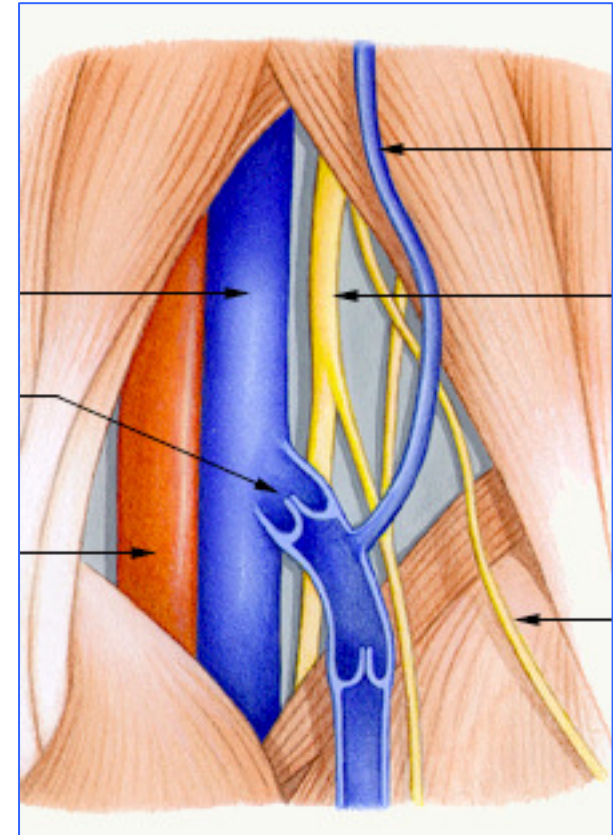


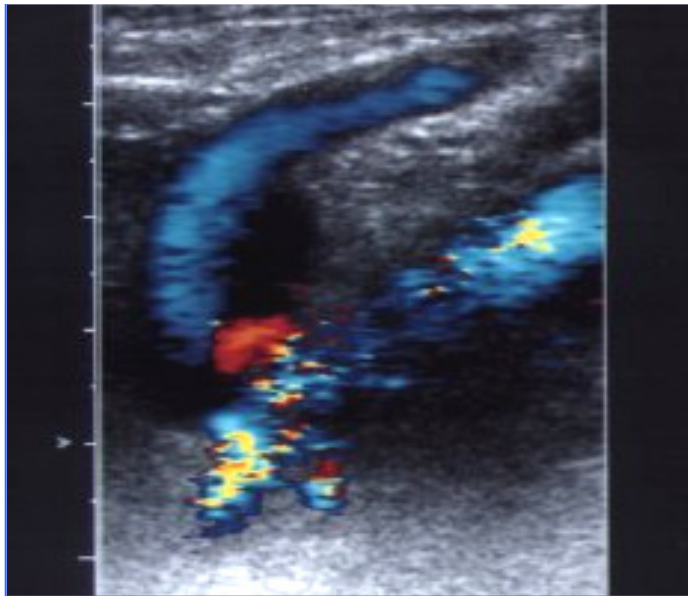
La Saphène externe / Petite Saphène

- **Crosse : variations retrouvées dans 50% des cas .(++++)**
- **Anastomose saphène-saphénienne : veine de Giacomini.**



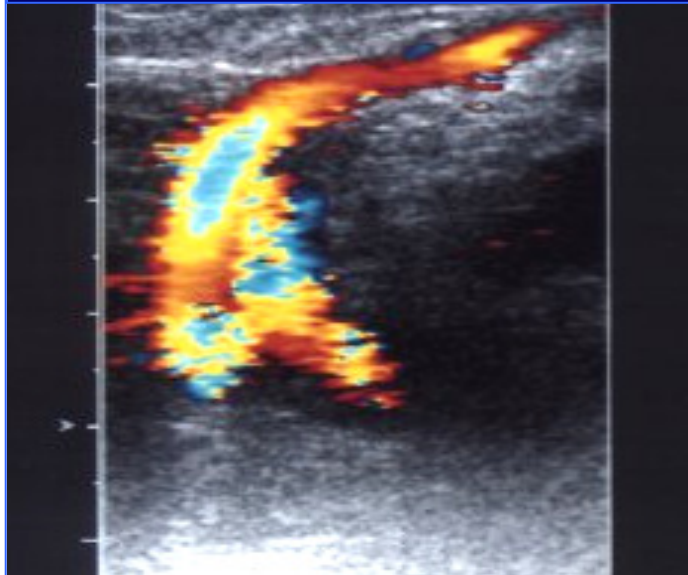
Giacomini





Flux

Reflux Petite Saphène

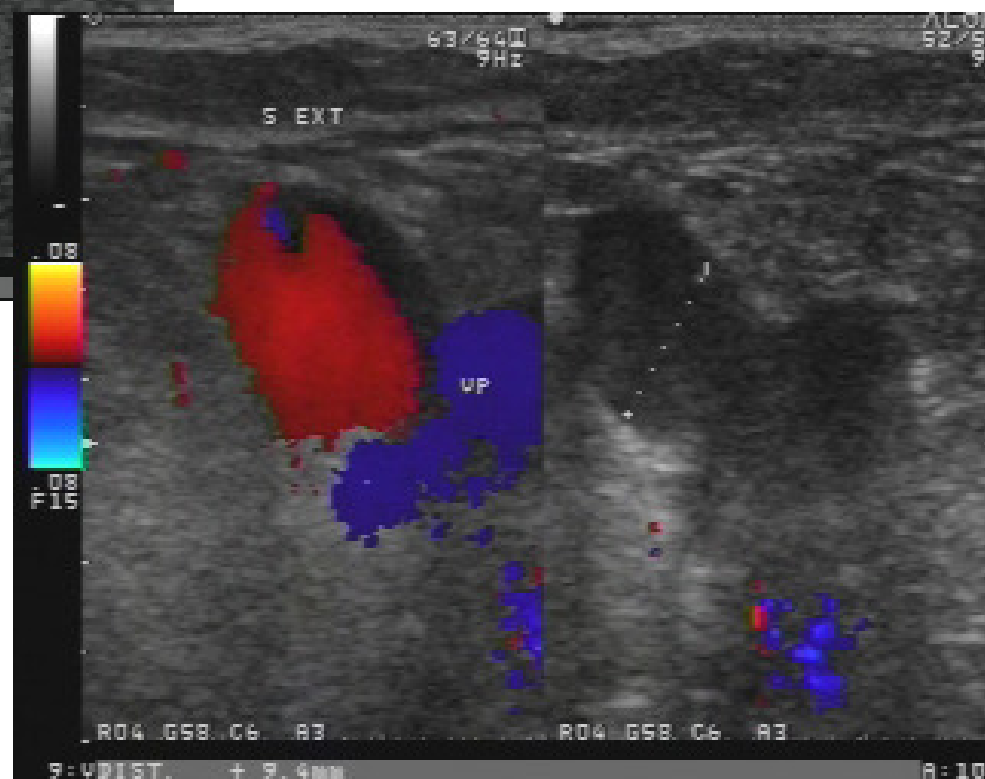
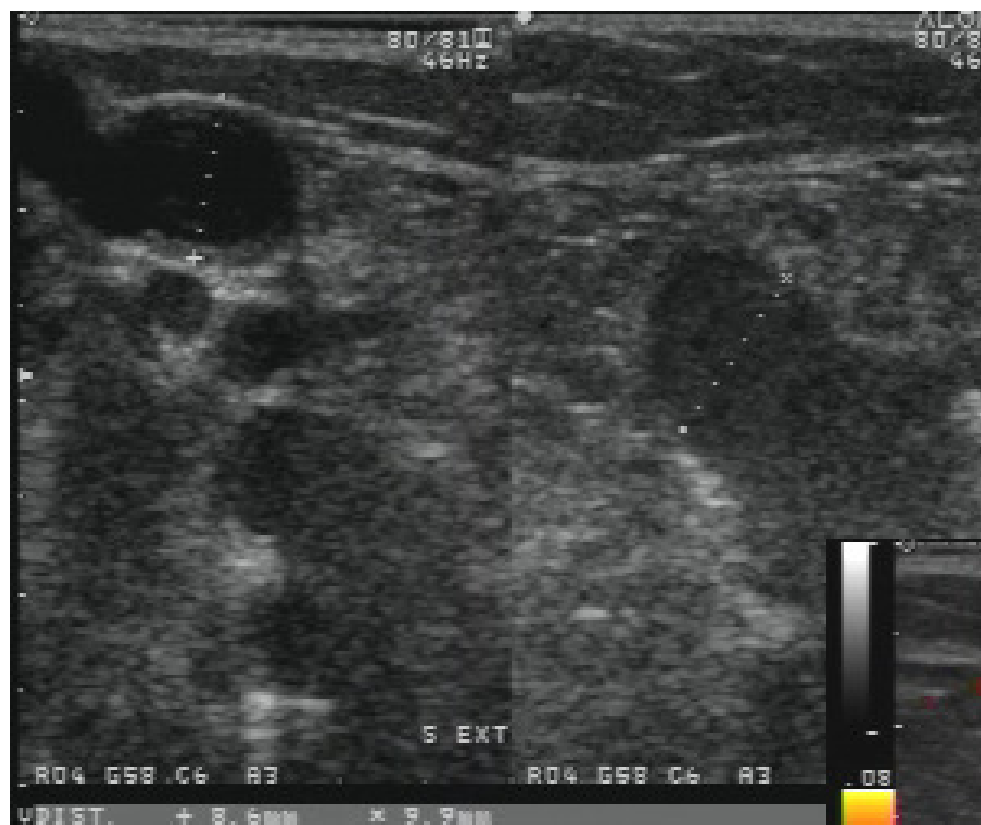


Reflux

Position



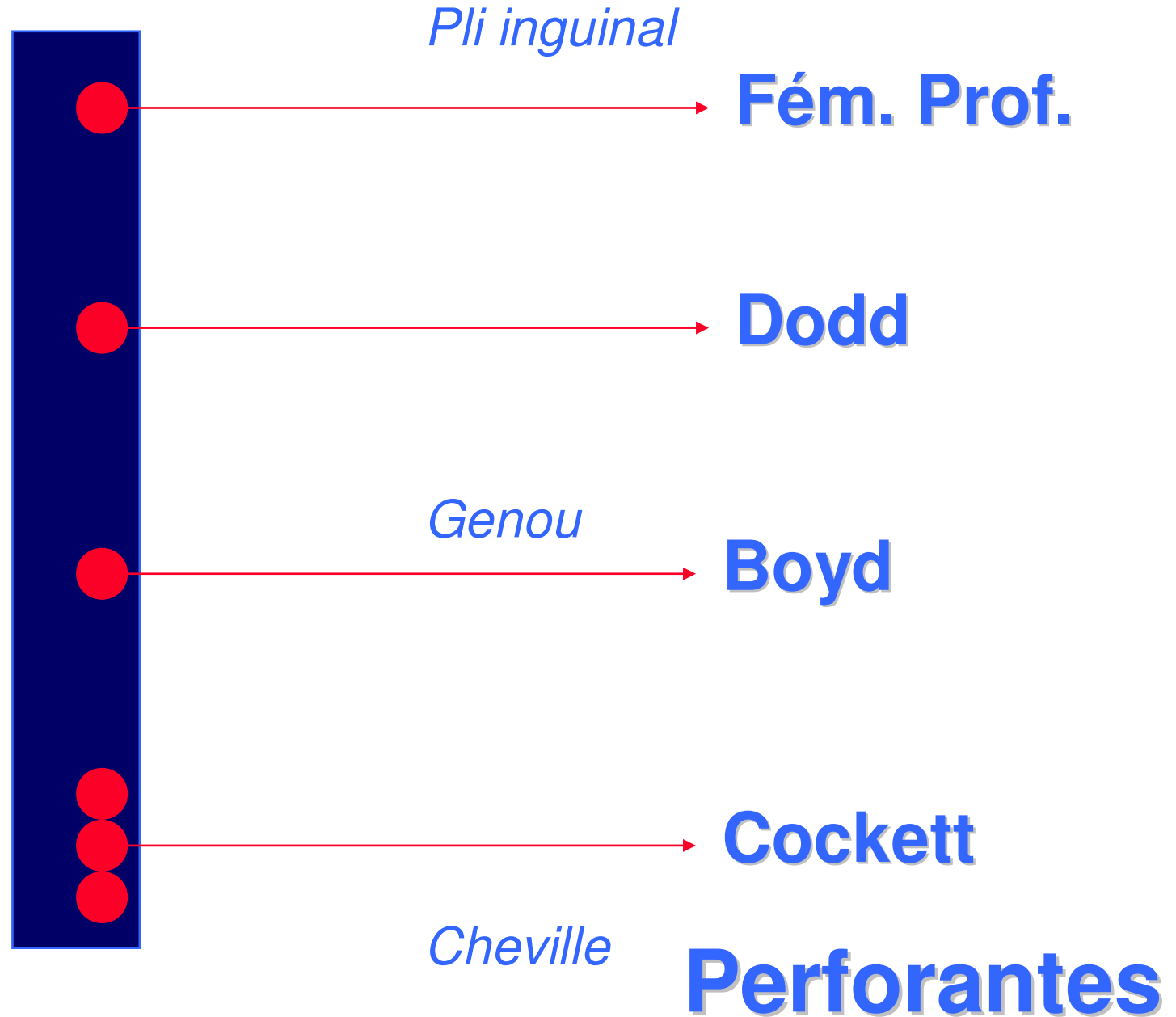
Petite saphène



Les Perforantes

- Voies de suppléance.
- Anastomoses entre veines superficielles et profondes qui perforent l'aponévrose.
- Leur flux va de la superficie à la profondeur.
- Système d'équilibration qui devient défaillant surtout en cas d'atteinte des veines profondes.

ANT.



Perforantes

Sous-Fessière *Pli fessier*

Fém.Prof.

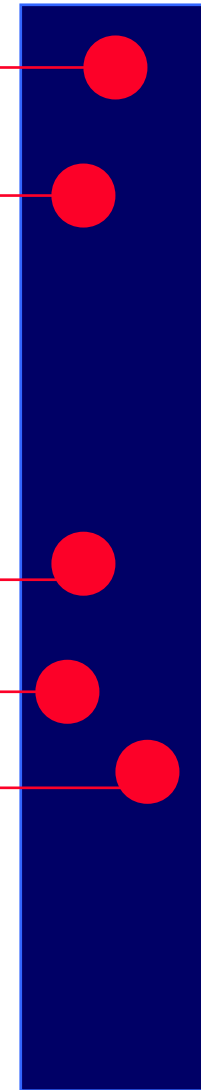
Fosse pop *Pli Poplité*

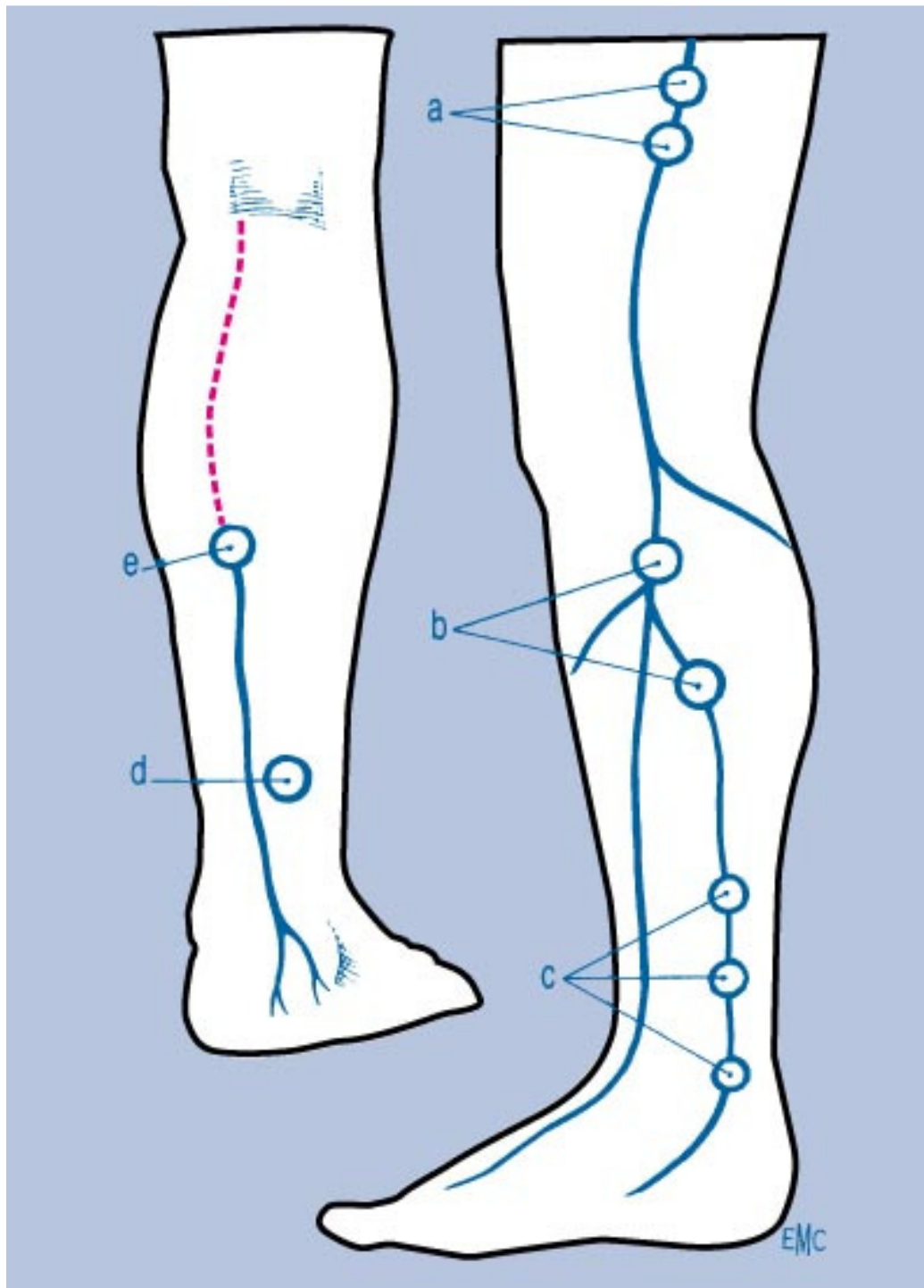
Péronière

Jumelle

Cheville

Post.

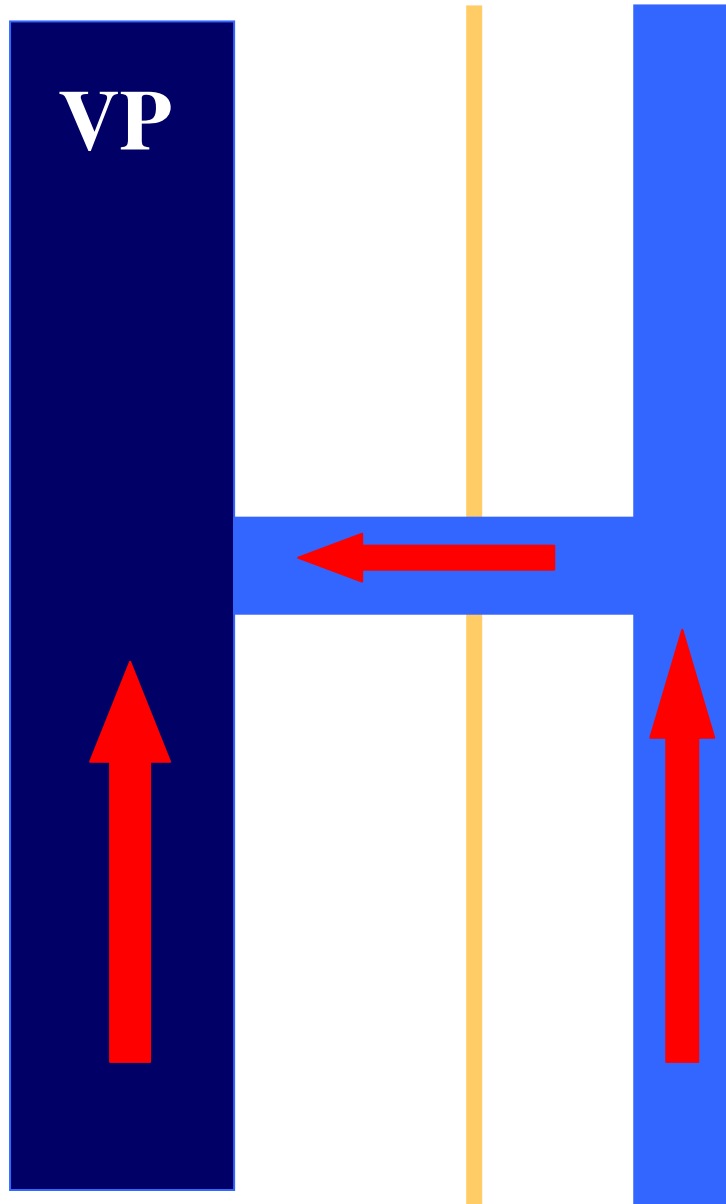




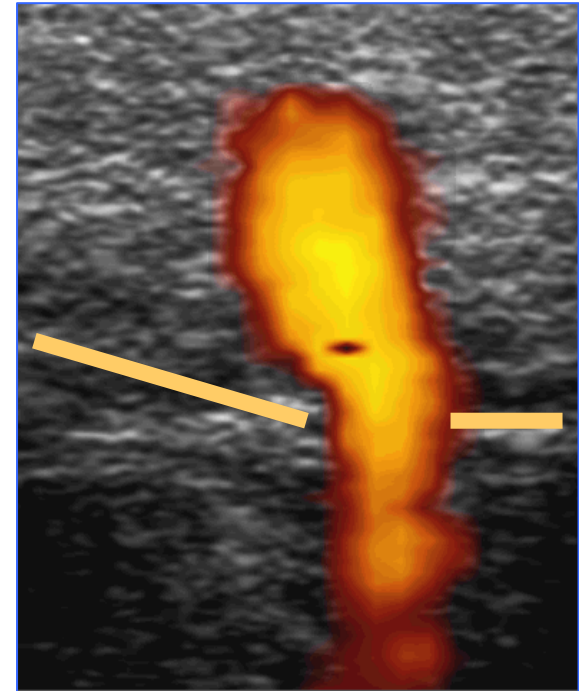
A : Dodd
B : Boyd
C : Cockett
D : Perf Ext de cheville
(VP/VSE)
E : Perf médiane post
(VJI/VSE)

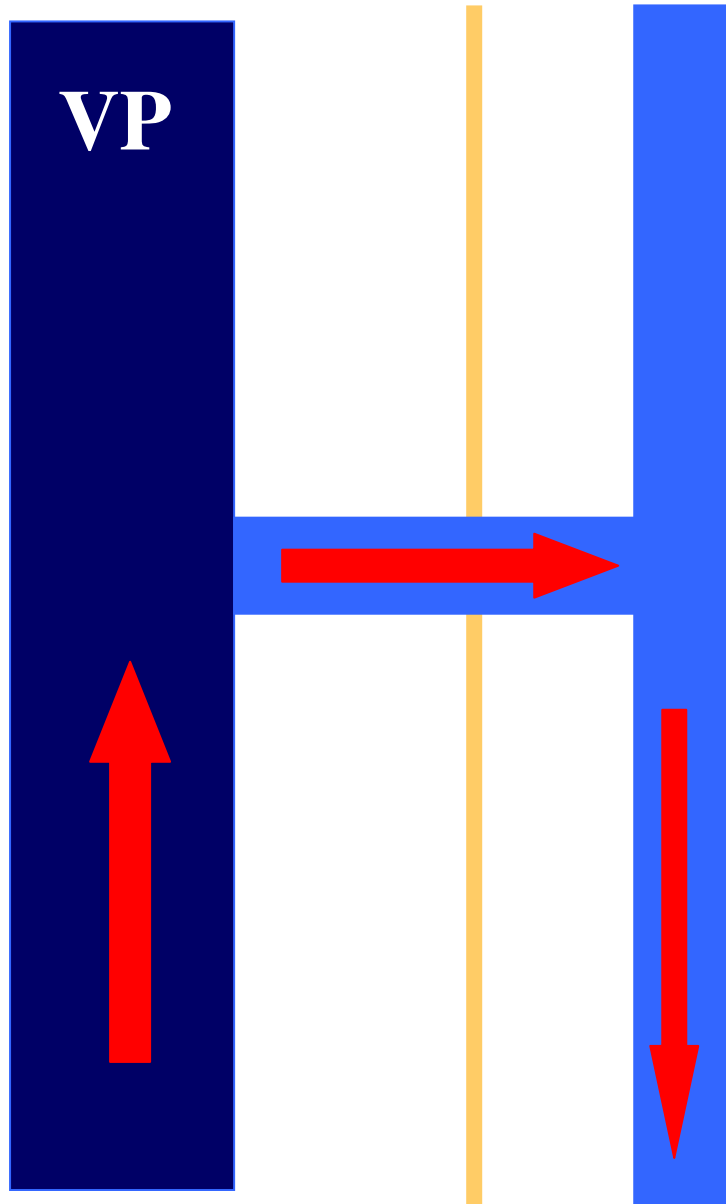
Perforantes : groupe de perforantes

- **Leur diamètre n ' est pas à prendre en considération**
- **Doppler Couleur : Examens de référence.**
- **Les perforantes incontournables :
Dodd, Boyd, Fosse poplitée, Cockett, Jumelles, fémorale prof., sous-fessière**

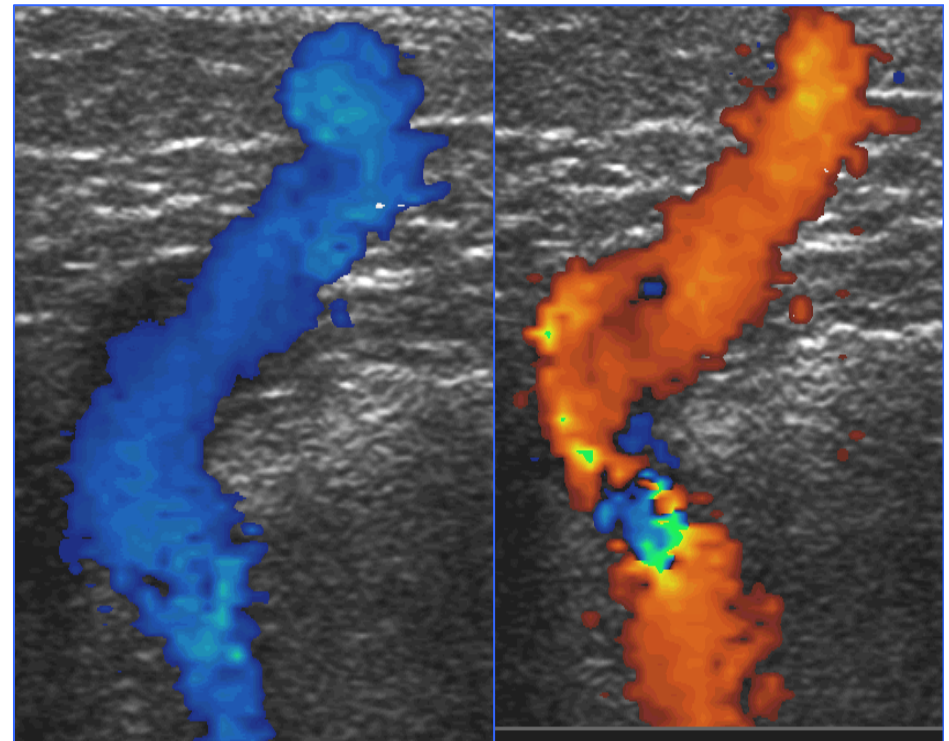


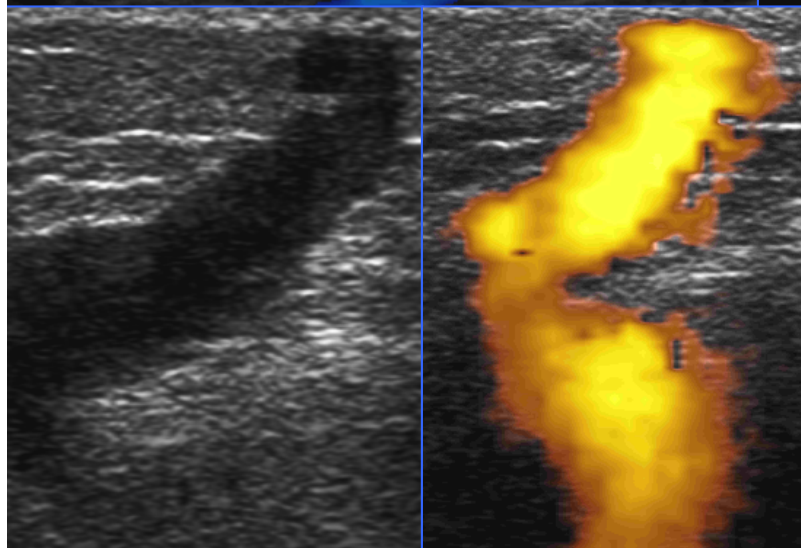
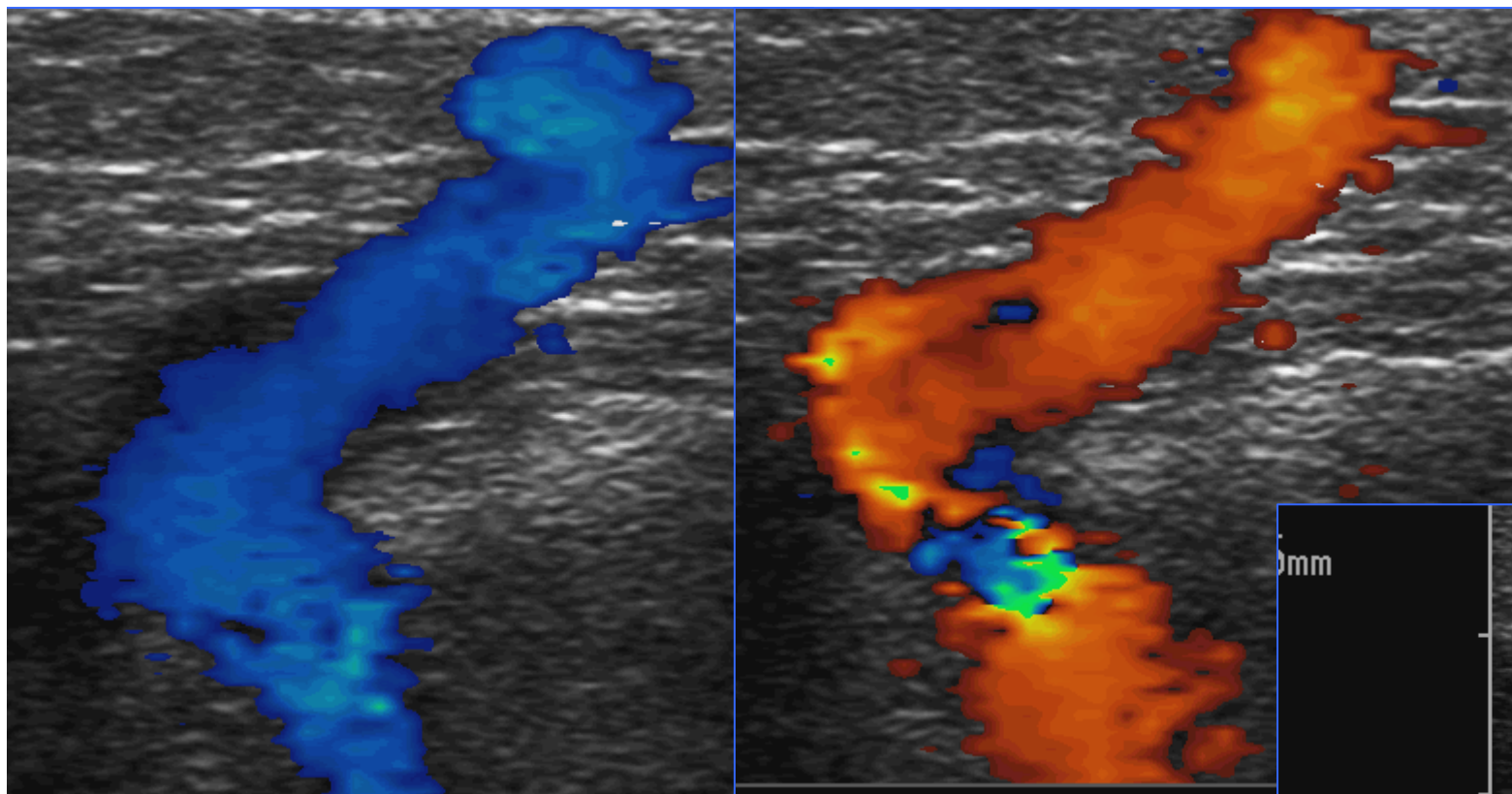
Perforante Directe





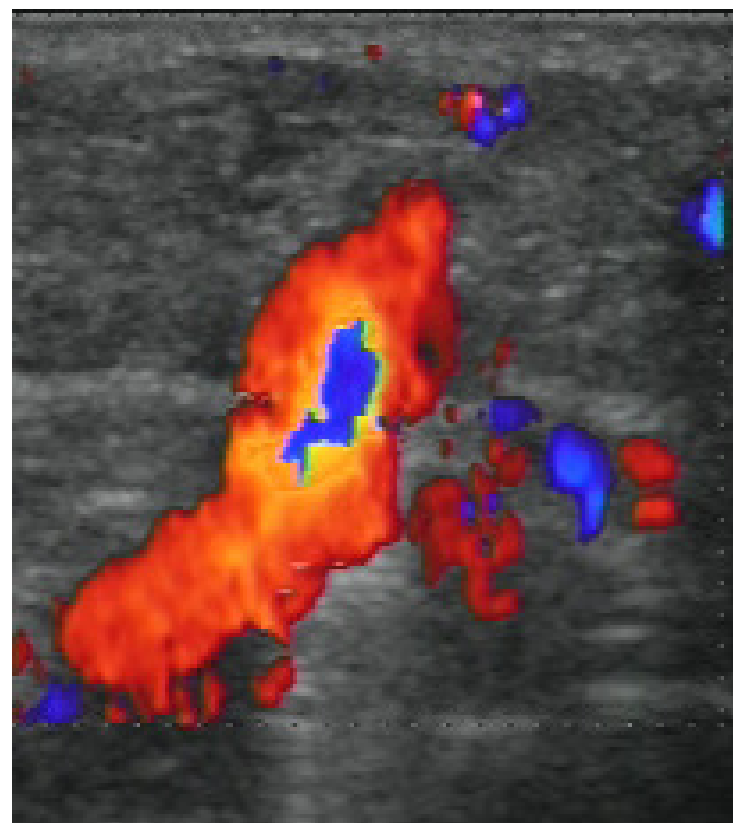
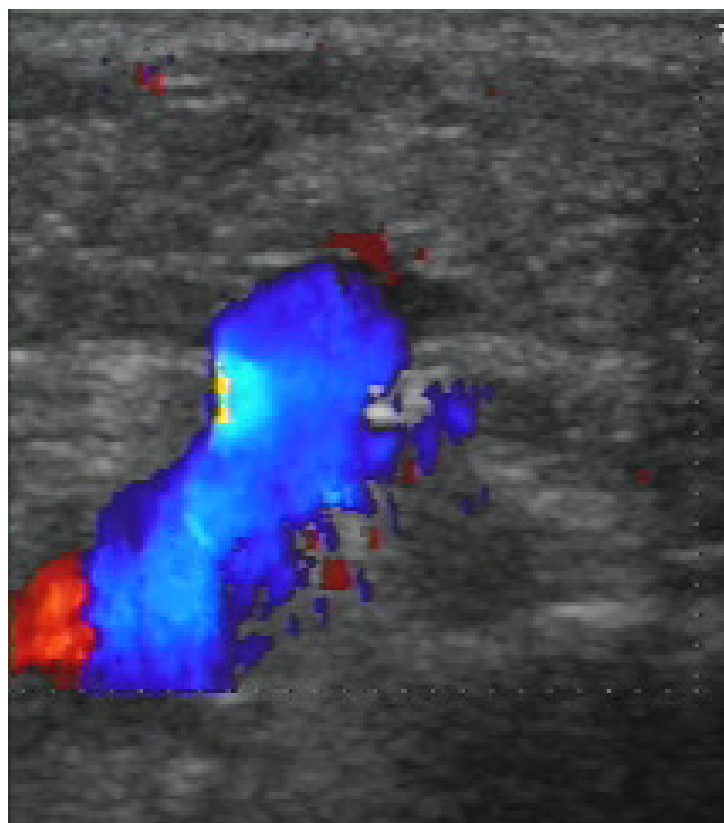
*Perforante Directe :
Reflux*



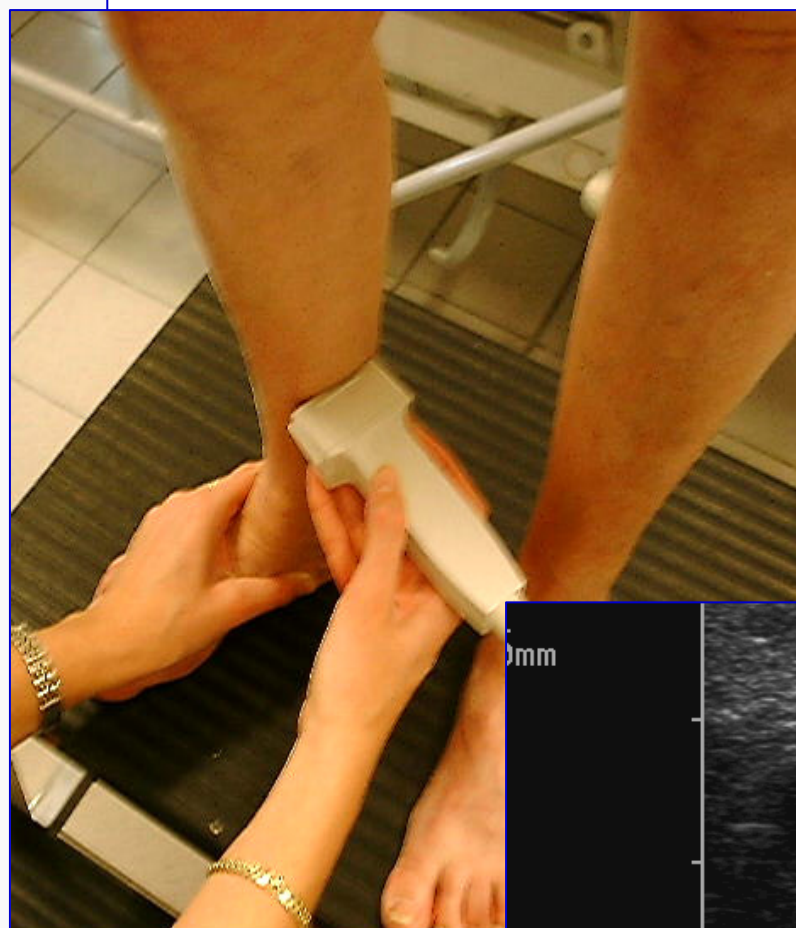
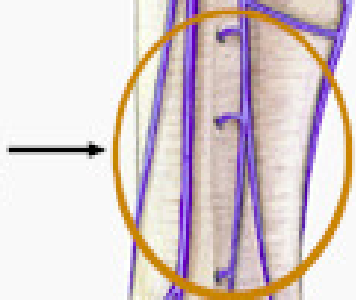


***Perforante
incontinente
(Cockett)***





Perforantes
de Cockett



Varices : cas particuliers

- 1. Varices du sportif : attention aux veines turgescences.
- 2. Varices et grossesse : pas de traitement définitif entre les grossesses, attention aux reflux occasionnels, bilan 3 mois après l'accouchement. Mais **PREVENTION** (+++++) durant la grossesse : contention, HV, phlébotrope (1), hydratation.